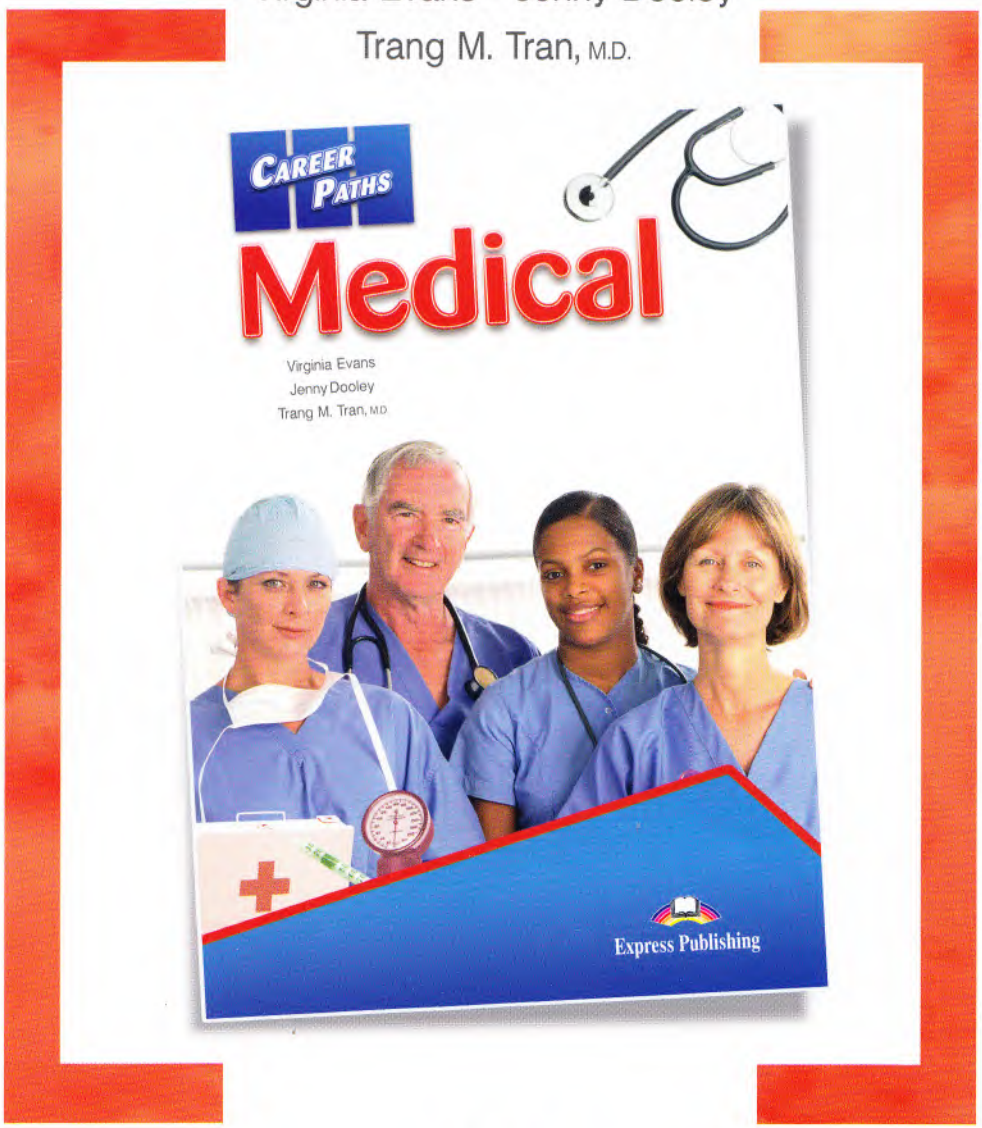
Teacher's Book

Virginia Evans - Jenny Dooley
Trang M. Tran, M.D.



Express Publishing

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CAREER PATHS

Medical

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Unit 1

1 Suggested Answers

1 Some different hospital department are pediatrics, radiology, obstetrics and emergency.
2 It is important to know what different departments do so that patients are sent to the correct ones to treat their problems.

2 1 T 2 T 3 F

3 1 E 2 D 3 A 4 C 5 B

4
1 pathology
2 emergency
3 radiology
4 pharmacy
5 surgery

5 Suggested Answer

The hospital has many departments for different patients. Pediatrics treats children while obstetrics treats pregnant women. Heart patients go to cardiology, while people with skin problems go to dermatology. Athletes may need to visit the orthopedics department.

6 1 B 2 A

7
1 in great health
2 do about
3 look at
4 on the third floor
5 radiology
6 pharmacy

8 Suggested Answer

A: Other than your wrist, you're in good health.
B: Okay, Doctor. Where should I go next?
A: I want you to go to radiology for some X-rays.
B: Where's that department?
A: It's on the fourth floor. Turn left after the dermatology department.
B: I'll go there right away.
A: But first, you should stop by the pharmacy. Your prescription will be ready soon.
B: That's on the second floor right?
A: Yes, next to the surgery department.

9 Suggested Answer

Mary Coburn
Hospital visit notes

First stop: The pharmacy
Location: It's on the second floor, next to the surgery room.
Reason: Refill my prescription.

Second stop: Radiology
Location: It's on the fourth floor. Turn left after dermatology.
Reason: Make appointment to look at rash.

Unit 2

1 Suggested Answers

1 Some people who work in a hospital include pharmacists, radiologists, general practitioners, nurses and lab technicians.
2 There are many special kinds of doctors including cardiologists, pediatricians or obstetricians, general practitioners and anesthesiologists.

2 1 T 2 F 3 F

3 1 C 2 B 3 A 4 D 5 E

4
1 radiologist
2 pediatrician
3 cardiologist
4 obstetrician
5 surgeon

5 Suggested Answer

Many different people work in the hospital. Dr. Sartin is a cardiologist and heart surgeon. Dr. Collins is a pediatrician, so he treats children.

6 1 C 2 B

7
1 for a second
2 over
3 a bit too high
4 thought so
5 cardiologist
6 know about

8 Suggested Answer

A: Could you come over here for a second?
B: Sure, what do you need?
A: Take a look at this X-ray.
B: Hmm ... that looks like a broken rib.
A: Yes, that's what I thought
B: Who's the radiologist?
A: Dr. Locke, it says here.
B: He should examine the patient. He must have missed this.
A: I agree. I'll let his office know to set up an appointment.
B: Good idea. Thanks for consulting me.

9 Suggested Answer

Heartland Hospital
Patient Consultation Form

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Patient: Martin Vasquez
Doctor Consulted: Dr. McNeil
Issue: The patient's chart showed a broken rib.
Solution: Mr. Vasquez's radiologist is Dr. Locke. I called his office to set up an appointment with the patient.

Unit 3

1 Suggested Answers

1 There are many common pieces of hospital equipment such as wheelchairs, gauze, syringes and pressure mattresses.
2 Syringes should be placed in a protective sharps container. Other waste such as gauze or latex gloves should be placed in a biohazard waste container.

2 1 F 2 F 3 F

3 1 D 2 C 3 E 4 B 5 A

4
1 latex gloves
2 oxygen tank
3 bedpans
4 wheelchair
5 pressure mattress

5 Suggested Answer

It is important to dispose of medical waste in a safe manner. Used syringes should be placed in a sharps container. Other medical waste such as gauze and used latex gloves should be placed in a biohazard waste container.

6 1 C 2 A

7
1 short on anything
2 we need some
3 Anything else
4 only a few
5 don't forget to
6 See you tomorrow

8 Suggested Answer

A: Have you done your supply checks for the day?
B: Yes, I just finished them up.
A: Are we short on anything?
B: Yes, we need to replace the bedpans in patient rooms 2 and 5.
A: Okay. Anything else?
B: We're almost all out of gauze.
A: Okay, I'll have to order some more. That reminds me, don't forget to empty the biohazard waste containers before you leave.
B: Will do. See you tomorrow.

9 Suggested Answer

Family Clinic
Supply Order Form
Exam Rooms: More bedpans are needed in rooms 2 and 5.
Storage Closet: There is almost no gauze left in the storage closet. More will need to be ordered.

Unit 4

1 Suggested Answers

1 Parts of the arm are connected by the elbow and parts of the leg are connected by the knee. The hands are connected to the arm by the wrist and the legs are connected to the feet by the ankles.
2 Some common injuries are sprained ankles, knees, and elbows. Shoulders and hips can be knocked out of place. Fingers and toes can be stubbed, jammed, or broken.

2 1 T 2 T 3 F

3
1 C 3 F 5 A
2 E 4 B 6 D

4
1 A ankle B shoulder
2 A knee B toe
3 A foot B hand

5 Suggested Answer

There are many common sports injuries. Ankles, knees, and elbows can be sprained from unnatural movements. Shoulders and hips can be dislocated. Fingers and toes can be jammed or broken.

6 1 D 2 C

7
1 What seems to be
2 does it hurt
3 All over
4 what happened
5 fell on my arm
6 anywhere else

8 Suggested Answer

A: What seems to be the problem Lisa?
B: My leg really hurts.
A: I see. Where exactly does it hurt?
B: Mainly around my ankle.
A: Okay. Do you know what happened to make it hurt?
B: I was running and fell down. My foot twisted.
A: Yeah, that's likely the problem. Does it hurt anywhere else?
B: No, just my leg
A: Okay. I think it's just an ankle sprain, but we'll take some X-rays to make sure.

9 Suggested Answer

Valley Medical Clinic
Medical Chart

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Patient: Lisa Jones
Symptoms: Her leg hurt, especially her ankle.
What happened: She was running and fell down.
Next steps: Lisa likely has an ankle sprain. I will take some X-rays to make sure her foot is not broken.

Unit 5

1 Suggested Answers

1 The major parts of the human torso are the back, the rib cage, the waste and the groin.
2 The major part of the torso that protects the body is the rib cage. This bone structure keeps the vital organs safe. Without it, our lungs, heart, and major arteries could be easily damaged.

2 1 C 2 B 3 D

3 1 G 3 H 5 F 7 A
2 E 4 C 6 B 8 D

4 1 A small B rib cage
2 A back B neck

5 Suggested Answer

The parts of the torso on the front of the body include the neck, the groin, the abdomen, and the chest, the waist and the rib cage.

6 1 F 2 F 3 F

7

1	brings you in	4	small
2	back	5	shoulder blades
3	Where does it hurt	6	bend over

8 Suggested Answer

A: Hi, Joe. What brings you in today?
B: My chest hurts.
A: All right. Where does it hurt exactly?
B: It's just above my rib cage.
A: Any pain near the neck?
B: No, that part doesn't hurt.
A: I see. Does it only hurt when you move your arms?
B: No, it hurts when I wake up in the morning too.
A: Okay, I'll take a look and see what I can find.

9 Suggested Answer

Dr. Jackson
Date: April 20

Patient: Joe Gayle
Reason for visit: Patient is feeling pain.
Areas patient feels pain: Patient feels pain in his chest, just above the rib cage.
When patient feels pain: Patient feels pain when he moves his arms and when he wakes up.

Unit 6

1 Suggested Answers

1 There are several parts of the face that sense things. The eyes sense light and provide vision. The nose senses smells, and the mouth and tongue sense taste.
2 The parts of the face that do not directly involve the senses are: the forehead, cheeks, eyebrows, lips, and jaw.

2 1 B 2 C 3 B

3 1 D 3 B 5 C 7 G
2 E 4 F 6 A

4 1 B 2 A 3 A

5 Suggested Answer

The patient's ears, eyes and eyebrows, are not injured.

6 1 F 2 F 3 T

7

1	she doing	5	Above or below
2	facial injuries	6	forehead
3	Where	7	left ear
4	left eye		

8 Suggested Answer

A: I saw the patient in room ten.
B: How is she doing?
A: She has several scrapes on her face. Some look serious.
B: Where exactly are the scrapes?
A: Two above the left eye. And a deep one on the left cheek, near the nose.
B: Is that all?
A: No, her lip is a bit swollen.
B: Okay, I'll take a look.

9 Suggested Answer

PATIENT MEDICAL FORM
Head: bump on the back of the head
Eyebrows: no injuries
Eyes: no injuries
Forehead: two scrapes above left eye
Jaw: no injuries
Cheek: deep cut on left cheek
Nose: broken
Ears: no injuries
Mouth: swollen lip

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Unit 7

1 Suggested Answers

1 Some parts of the respiratory system are the bronchial tubes and the lungs.
2 Two common ailments of the respiratory system are asthma, which is when the bronchial tubes are inflamed and emphysema is where the alveoli lose their shape and stop working properly.

2 1 B 2 D 3 A

3
1 bronchial tubes
2 asthma
3 exhale
4 lungs
5 Oxygen

4
1 inhale
2 breathe
3 alveoli
4 emphysema
5 respiratory system

5 Suggested Answer

Emphysema is a disease that affects the respiratory system. It damages the alveoli and causes them to lose their shape when the infected person exhales.

6 1 T 2 F 3 F

7
1 what did you find
2 sorry to say
3 asthma
4 condition
5 treatments
6 breathe

8 Suggested Answer

A: Good morning Sally. I've received the test results.
B: What did you find doctor?
A: I'm sorry to tell you, you have asthma.
B: Oh, dear. Can it be treated?
A: Yes. For one, I can give you a prescription for some medicine.
B: So I'll take pills?
A: Actually, no. You'll inhale it.

9 Suggested Answer

Doctor: Clarence Williams MD
Prescription Note

Patient Name: Sally McNeal **Date:** 11/15
Diagnosis: Patient has asthma.
Prescription: 25 ml medicine inhaler. Use inhaler 2x daily and as needed.

Unit 8

1 Suggested Answers

1 The main part of the circulatory system is the heart.
2 Blood flows through the body to the capillaries and then back again.

2 1 F 2 T 3 F

3 1 D 2 E 3 C 4 B 5 A

4
1 circulatory system
2 aorta
3 atrium
4 vena cava
5 heart

5 Suggested Answer

Blood picks up oxygen when it travels through the pulmonary artery.

6 1 B 2 C

7
1 heart attack
2 gets blocked
3 doesn't sound good
4 When that happens
5 symptoms
6 Sudden chest pains

8 Suggested Answer

A: Your smoking habit puts you at increased risk for heart failure.
B: Could you explain what that means, exactly?
A: Certainly. It's often caused by a heart attack. Basically, it just means the heart cannot supply enough blood flow to meet the body's needs.
B: That doesn't sound good.
A: No, it's not. It is a serious condition that could lead to long-term disability or death.
B: Oh my. What are the symptoms?
A: Call the hospital immediately if your leg swells or if you have trouble breathing.
B: I will. Thank you, Doctor.

9 Suggested Answer

Heartland Hospital
Heart Attack Information

Risk factor: Smoking places a person at an increased risk of heart failure.
What happens: During a heart attack, the heart cannot get enough blood flowing. This can eventually cause permanent damage or death.
Symptoms: Leg swelling or trouble breathing can be signs of a heart attack. Call the hospital immediately if you have these symptoms.

Unit 9

1 Suggested Answers

1 The main parts of the digestive system are the stomach, the esophagus and the large and small intestines.

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2 There are many possible problems that can occur in the digestive system. Some are one-time problems, such as appendicitis. Others are more long term, such as ulcers.

2 1 C 2 D 3 B

3 1 B 2 D 3 C 4 A 5 E

4
1 A large intestine B small intestine
2 A ulcer B colon

5 Suggested Answer

Problems in the digestive system can appear anywhere. Problems can appear in the esophagus, the stomach, the appendix, or the colon. Each problem is different. Problems include ulcers in the stomach and inflammation of the appendix.

6 1 F 2 T 3 F

7
1 same pain
2 have to do
3 that's necessary
4 good idea
5 needs to be done
6 Better safe

8 Suggested Answer

A: Hi, Doctor Allen.
B: Hello Mr. Harris. You're still having the stomach pain?
A: Yeah. I've had it for about a week.
B: All right. We're probably going to have to do a colonoscopy.
A: Really? Do you think that's necessary?
B: Yes. We should check for any problems.
A: Okay. What does it involve?
B: We'll insert a fiber optic camera into the rectum. While the camera is being withdrawn, we'll find the problem.
A: All right. Better safe than sorry I guess.

9 Suggested Answer

Date: July 14
Patient Name: Jim Harris
Description of problem: Stomach pain lasting over a week. Ulcer suspected.
Suggestions: Patient should come back next week for a colonoscopy.

Unit 10

1 Suggested Answers

1 Many kinds of small injuries can be helped through basic first aid. These include minor cuts, burns, and lacerations. People trained in first aid can also assist in dealing with more serious conditions. Such conditions include the lack of a pulse or breathing.
2 Some common first aid techniques are CPR and rescue breathing, which can be performed on victims who have stopped breathing or lost their pulse.

2 1 F 2 T 3 F

3
1 D 3 A 5 C
2 F 4 E 6 B

4
1 assess
2 first aid
3 pressure
4 bandage
5 wound

5 Suggested Answer

First, it is always important to call for medical help in an emergency. Several first aid techniques can be applied before help arrives. These include applying pressure to wounds and cleaning and bandaging them. CPR and rescue breathing are other important techniques.

6 1 A 2 A

7
1 What's troubling you
2 really hurts
3 take a look
4 first aid
5 ran some
6 should be fine

8 Suggested Answer

A: Hello. What's troubling you today?
B: My arm really hurts.
A: Let me take a look at it. Yes, you have a nasty cut there. What happened?
B: I fell on some sharp rocks.
A: I see. Did you get any first aid?
B: Yes. Someone applied pressure until the bleeding stopped.
A: It looks like it helped. I'll clean it and put a few stitches in it. You'll be fine in a few days.
B: Thank you, Doctor. I'll be more careful from now on!

9 Suggested Answer

Family Clinic
Patient Chart

Patient: Bill Johnson
Symptoms: The patient's arm has a large cut on it.
Cause: The patient fell on some rocks.
Treatment: Someone applied pressure until the bleeding stopped.

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Unit 11

1 Suggested Answers

1 Some common medical abbreviations include T, HR and BP.
2 To save time filling out prescriptions and exam sheets doctors and nurses often abbreviate common words and phrases such as o/e as an abbreviation for "on examination," and XR for x-ray.

2 1 F 2 F 3 F

3 1 E 3 C 5 A 7 G
2 D 4 B 6 F

4 1 A XR B SOB
2 A Rx B HTN

5 Suggested Answer

The doctor's note said that the patient c/o SOB. This is an abbreviated way of saying the patient complains of shortness of breath.

6 1 A 2 C

7

1	read his writing	4	SOB
2	Let's see	5	diagnosis
3	c/o	6	HTN

8 Suggested Answer

A: Can you look at this note from Doctor Smith? I can't read what it says.
B: Sure. It says o/e patient did not appear anxious.
A: Okay. And this is the diagnosis?
B: Yes. It says the patient has hypertension.
A: Oh yeah. Now I see it. It says HTN.

9 Suggested Answer

Dr. Smith
Patient Name: Gabriel Simmons
Date: 12/09

T: 98.6 F
HR: 82 BPM
BP: 142/93
Respiration: normal

Dx: hypertension

Unit 12

1 Suggested Answers

1 Some metric units used to measure medicine doses include milliliters, grams, milligrams and cubic centimeters.
2 Familiarity with measurements is important because one needs to give the proper amount of medication to a patient. Too much medication could be harmful, as could too little medication. Also, it is important to be aware of how much medication someone takes in a given period of time.

2 1 C 2 D 3 D

3 1 B 3 A 5 E
2 C 4 F 6 D

4

1	A	drops	B	milliliters
2	A	ounces	B	grams

5 Suggested Answer

Different dosages are measured out in milliequivalents, cubic centimeters, and grams.

6 1 T 2 T 3 F

7

1	have a question	4	bad infection
2	What's up	5	milligrams
3	a little high	6	goes away

8 Suggested Answer

A: I have a question about Mr. Henry's medication.
B: What's up?
A: This note calls for six drops in each eye three times a day. That seems a little high.
B: That's right. They're higher because he has an eye infection.
A: So that's why there's this new antibiotic here?
B: Right. He'll take sixty milligrams twice a day.
A: All right. That's all I wanted to know.
B: Okay. Thanks for checking.

9 Suggested Answer

Name: Henry, M.
Medication 1: Eyedrops
Dose: six gtt
Number of doses per day: Three per day

Medication 2: Antibiotics
Dose: 60 mg
Number of doses per day: Twice per day

Unit 13

1 Suggested Answers

1 There are many different routes of administration for medicine. They may be taken P.O. or topically. They may also be delivered I.V. or sublingually.

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2 Medical professionals must be prepared for any patient and situation. The preferred route of administration for a medicine may not be available. Knowing different ways to give medicine could help save a patient's life in an emergency.

2 1 F 2 F 3 T

3
1 F 3 A 5 E
2 C 4 D 6 B

4
1 route of administration 3 administer
2 intramuscular injection 4 dosage

5 Suggested Answer

Ms. Gates received medicine in several different ways. First she received a topical ointment. Later, she needed more serious medicine through an I.V. and intramuscular injections. When she was discharged she received antibiotic pills to be taken P.O.

6 1 D 2 D

7
1 before I head out 4 would work better
2 twice daily 5 He'll want to
3 Should that be 6 chew tablets

8 Suggested Answer

A: Let's review these patients' meds before I head out.
B: Sure thing, Doctor.
A: Ms. Smith should receive 100 mg of steroids twice daily.
B: Should that be administered through an IM or I.V.?
A: Use an I.V. so the administration is slower. Second, Mr. Jones should take his heart pills every six hours.
B: Okay. Is he able to swallow them?
A: Yes, standard P.O. administration is fine.
B: Got it. Thanks, Doctor. Have a good night.

9 Suggested Answer

Heartland Hospital
Patient Medications

Patient: Claire Smith
Medication: 100 mg of steroids twice daily.
Route of administration: I.V. injection

Patient: Bill Jones
Medication: heart pills, every six hours.
Route of administration: standard P.O. administration.

Unit 14

1 Suggested Answers

1 There are many different frequencies for administering medicine. A drug might be given twice a day (BID), three times a day (TID), or four times a day (QID). Some should be taken before or after eating - ac for before meals and pc for after meals.
2 Medical professionals should know all the abbreviations for dosing frequencies to ensure medicines work in the best way possible.

2 1 T 2 F 3 T

3
1 E 3 D 5 A
2 B 4 C 6 F

4
1 stat 3 ac 5 pc
2 qhs 4 qh

5 Suggested Answer

The patients need their medications at many different times. Mr. Foster needs his heart pills TID. Ms. Richards can have ibuprofen PRN. Ms. Edmonds needs steroid injections BID. Mr. Davies needs injections QID.

6 1 A 2 D

7
1 What does he need 4 should be taken
2 antiviral pills 5 Anything else
3 how often 6 not more than

8 Suggested Answer

A: Let's go over the new patient's medications.
B: Okay, Doctor. What does he need?
A: He needs a 50 cc steroid injection.
B: Sure. And how often should he get those?
A: They should be given BID.
B: I understand. Anything else?
A: Give him 300 mg of this antibiotic every night, qhs.
B: Got it. I'll add that to his chart.

9 Suggested Answer

Heartland Hospital
Patient Medication Chart

Patient: Ed Walker

Medication 1: 50 cc steroid injection.
Frequency: They should be given BID.

Medication 2: 300 mg antibiotics.
Frequency: He should be given them every night, qhs.

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Unit 15

1 Suggested Answers

1 Cleanliness is important in a hospital to stop the spread of contagious viruses and drug resistant bacteria. If these are transmitted they can cause many infections in patients.

2 To avoid infections antimicrobial soap must be used. Antibiotic drugs should be stored correctly and infected patients should be put in quarantine. Finally, face masks must be worn in quarantine areas.

2 1 C 2 C 3 A

3 1 C 3 E 5 A 7 G
2 D 4 B 6 F

4 1 A antimicrobial B biohazard
2 A disinfectant B virus

5 Suggested Answer

Illnesses are passed from person to person by the transmission of infectious microbes. This often occurs through the membranes of the nose and mouth. Wearing a face mask can help prevent these transmissions from occurring.

6 1 F 2 T 3 F

7 1 a day or two 4 antimicrobial
2 give it 5 anything else
3 highly contagious 6 facemask

8 Suggested Answer

A: Well Mrs. Simms, you have the flu. It's a fairly common strain. You should recover in a day or two.
B: Okay. I hope I don't give it to my spouse.
A: It is a highly contagious illness. Chances are he will get it.
B: Is there anything I can do to stop it?
A: Just make sure to wash your hands regularly. And use antimicrobial soap.
B: I will. Thanks Doctor.

9 Suggested Answer

Patient: Alina Santos
Diagnosis: Ms. Santos has the flu.
Concerns: Ms. Santos has a spouse at home that she does not want to transfer the virus to.
Treatment Suggestions: I advised Ms. Santos to use antimicrobial soap.

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Audioscripts

Unit 1

Doctor (M): Well, Mary, other than that rash, you're in great health.
Patient (F): Okay, Doctor. What should I do about it?
Doctor: I want you to go to dermatology. Make an appointment for them to look at it.
Patient: Where's that department?
Doctor: It's on the third floor. Turn right when you pass the radiology department.
Patient: Great, I'll head up there now.
Doctor: But first, you should refill your prescription at the pharmacy.
Patient: The pharmacy is downstairs, right?
Doctor: Yes, next to the emergency room.

Unit 2

Nurse (F): Dr. McNeil, could you come over here for a second?
Doctor (M): Sure, Carol. What do you need?
Nurse: I just took Ms. Greyson's blood pressure. It's one fifty over ninety-two.
Doctor: Hmm. That's a bit too high.
Nurse: Yes, I thought so, too.
Doctor: Who's her cardiologist?
Nurse: Her chart says it's Dr. Sartin.
Doctor: He'll want to know about this.
Nurse: I'll let his office know to set up an appointment.
Doctor: Good idea. Thanks for consulting me.

Unit 3

Nurse 1 (F): Gary, have you done your supply checks for the day?
Nurse 2 (M): Yes, I just finished them up.
Nurse 1: Are we short on anything?
Nurse 2: Yes, we need some more latex gloves in exam rooms 1 and 3.
Nurse 1: Okay. Anything else?
Nurse 2: There are only a few syringes left in the storage closet.
Nurse 1: Okay, I'll have to order some of those. That reminds me, don't forget to empty the sharps containers before you leave.
Nurse 2: Will do. See you tomorrow, Carol.

Unit 4

Doctor (F): What seems to be the problem, Tommy?
Patient (M): My arm really hurts.
Doctor: I see. Where exactly does it hurt?
Patient: All over, but especially the elbow.
Doctor: Okay. Do you know what happened to make it hurt?
Patient: I was playing soccer and fell on my arm.
Doctor: Yeah, that's likely the problem. Does it hurt anywhere else?
Patient: No, just my arm.
Doctor: Okay. I think it's just an elbow sprain, but we'll take some X-rays to make sure.

Unit 5

Doctor (M): Hi, Jessica. What brings you in today?
Patient (F): My back hurts.
Doctor: All right. Where does it hurt exactly?
Patient: It's just the small of my back.
Doctor: Any pain near the shoulder blades or neck?
Patient: No, that part doesn't hurt.
Doctor: I see. Does it only hurt when you bend over?
Patient: No, it hurts when I wake up in the morning too.
Doctor: Okay, I'll take a look and see what I can find.

Unit 6

Nurse (M): Hi, Dr. Stevens. I just saw the patient in room seven.
Doctor (F): Ah, yes. How is she doing?
Nurse: She has several facial injuries. Most are just scrapes.
Doctor: Okay. Where are the scrapes exactly?
Nurse: There are a few above the left eye.
Doctor: Above or below the eyebrow?
Nurse: Above the eyebrow. On the forehead.
Doctor: I see. Is that all?
Nurse: No. There is a deep wound near her left ear that looks serious. She may need stitches.
Doctor: Thanks, Ben. I'll take a look.

Unit 7

Doctor (M): Good morning, Mrs. Simmons. I've taken a look at your chest x-rays.
Patient (F): Yes. And what did you find?
Doctor: I'm sorry to say, it appears to be emphysema.
Patient: Oh, really? I thought it was just my asthma.
Doctor: Well, you've had asthma for years. But this is an entirely new condition.
Patient: Can it be cured?
Doctor: Unfortunately, emphysema is incurable. But there are treatments that can slow its progression.
Patient: I hope they work. I just want to be able to breathe well.

Unit 8

Doctor (F): Mr. Robinson, your blood pressure is really high. You're at increased risk for a heart attack.
Patient (M): Could you explain exactly what happens during a heart attack?
Doctor: Certainly. Usually, an artery inside the heart gets blocked by fatty acids.

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Patient: That doesn't sound good.
Doctor: No, it's not. When that happens, blood can't get through. That can lead to permanent damage or death.
Patient: Oh my. What are the symptoms?
Doctor: Sudden chest pains or difficulty breathing. Call the hospital immediately if you have them.
Patient: I will. Thank you, Doctor.

Unit 9

Doctor (F): Hi, Mr. Harvey. You're here because of the stomach pain. Is that right?
Patient (M): Yeah. It's the same pain as a week ago.
Doctor: All right. You know, we're probably going to have to do a colonoscopy.
Patient: Really? Do you think that's necessary?
Doctor: It's a good idea if you're having persistent lower abdominal pains. We should check for any problems.
Patient: Okay. You have me convinced. What needs to be done?
Doctor: Briefly, we'll insert a fiber optic camera into the rectum. While the camera is being withdrawn, we'll find the problem.
Patient: All right. Better safe than sorry.

Unit 10

Doctor (F): Hello, Bill. What's troubling you today?
Patient (M): My finger really hurts.
Doctor: Let me take a look at it. Yes, it's all red with a blister. What happened?
Patient: I touched a hot stove.
Doctor: I see. Did anyone provide first aid?
Patient: Yeah, I ran some cold water over it.
Doctor: Well, it probably helped. It should be fine in a few days. Just put aloe vera on it.
Patient: Thank you, Doctor. I'll try to be more careful.

Unit 11

Nurse 1 (M): Hey, Susan. Can you take a look at these notes from Dr. Downing? I just can't read his writing.
Nurse 2 (F): It is pretty messy sometimes. Let's see.
Nurse 1: Look. Is this c/o or o/e?
Nurse 2: It's hard to tell. But I think it says, "patient c/o SOB."
Nurse 1: Ah. That makes sense. I can see it now. And this is the diagnosis here?
Nurse 2: Yes. It says, Dx HTN.
Nurse 1: So the patient has hypertension?
Nurse 2: That's right.

Unit 12

Nurse (F): Hey, Mark. I have a question about Mr. Paul's medication.
Head Nurse (M): Yes Nancy? What's up?
Nurse: This note calls for three drops in each eye twice a day. That seems a little high.
Head Nurse: That's right. They're a little higher than usual. Mr. Paul has a bad infection.
Nurse: Oh. So that's why there's this new antibiotic here. He'll take fifty milligrams once a day by mouth?
Head Nurse: Right. He'll take it until the eye infection goes away.
Nurse: All right. That's all I wanted to know.
Head Nurse: Okay. Thanks for checking with me.

Unit 13

Doctor (M): Cindy, let's review these patients' meds before I head out.
Nurse (F): Sure thing, Doctor.
Doctor: Ms. Fulton should receive 150 mg of antibiotics twice daily.
Nurse: Should that be administered P.O. or through an injection?
Doctor: An IM would work better. Second, Mr. Brown can have aspirin if he asks for it, but not more than 600 mg per day.
Nurse: Okay. He'll want to take that sublingually, right?
Doctor: Yes, he can't chew tablets.
Nurse: Got it. Thanks, Doctor. Have a good night.

Unit 14

Doctor (M): Sharon, Mr. Walker is a new patient. Let's review his meds.
Nurse (F): Okay, Doctor. What does he need?
Doctor: He needs a 150mg dosage of these antiviral pills.
Nurse: Sure. And how often should he get those?
Doctor: They should be taken TID. Preferably ac, before they bring his food.
Nurse: I understand. Anything else?
Doctor: He can also have two tablets of ibuprofen PRN for his pain, but not more than 800mg per day.
Nurse: Got it. I'll add it to his chart.

Unit 15

Doctor (F): Well Mr. Larson, you have a cold. It's a fairly common strain. You should recover in a day or two.
Patient (M): Okay. I hope I don't give it to my granddaughter.
Doctor: It is a highly contagious illness. Do you live in the same house?
Patient: Yes. She's visiting for the holidays.
Doctor: Well, make sure to wash your hands regularly. And use antimicrobial soap.
Patient: I will. Is there anything else I can do?
Doctor: You could wear a facemask for a few days. I'll give you a few to take home.

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Unit 1

1 Suggested Answers

1 Blood donation saves the lives of many people every day. Hospitals and other medical facilities are always in need of blood. When patients bleed heavily as a result of an injury or surgery, they need transfusions to replenish the lost blood.
2 There are four different blood types: Type A, Type B, Type AB and Type O. Only Type O can be given to people with all different types.

2 1 T 2 F 3 F

3

1	transfusion	4	Type O
2	Type AB	5	compatible
3	universal donor		

4 **Blood Parts:** plasma, red blood cell
Blood Groups: Type A, Type B
Stop Blood Loss: platelet, clot

5 Suggested Answer

The wrong blood type can cause the recipient's blood to clot and might even lead to death.

6 1 C 2 A

7

1	Hold still	4	Types O or B
2	transfusions	5	the wrong type
3	anyone might	6	very dangerous

8 Suggested Answer

A: We're almost done taking your blood Ms. Yuang.
B: Okay. So, could anyone receive my blood?
A: No. Your blood is Type B, which means that it's only compatible with other people who have Types B and AB. We can't give your blood to someone with Type O or A.
B: What happens if someone gets a transfusion with the wrong type?
A: That can be very dangerous, or even fatal.
B: You must have to be careful about labeling the blood.
A: That's right. We keep careful records so no one gets the wrong blood type.

9 Suggested Answer

Name: Ms. Yuang
Blood Type: Type B
Eligible Recipients: Patients with Types B and AB can receive this donor's blood.
Ineligible Recipients: Patients with Types O and A cannot receive this donor's blood.

Unit 2

1 Suggested Answers

1 Bones are a fundamental part of the body's makeup. The skeleton forms the basic shape and structure. The hard, outer part keeps the body's core and limbs strong and rigid. The marrow on the inside produces cells that contribute to healthy immune function.
2 There are several different types of fracture. Some of these include compound fractures, impacted fractures and comminuted fractures.

2 1 F 2 T 3 F

3

1	D	3	E	5	A	7	B
2	F	4	C	6	G		

4 1 B 2 B 3 A

5 Suggested Answer

A stress fracture is a minor crack that does not require repositioning of the bone. A displaced bone must be repositioned, otherwise the bone will be in the wrong place when it heals.

6 1 B 2 B

7

1	tell me what	4	stress fracture
2	appears to be	5	take a closer look
3	the type of	6	comminuted

8 Suggested Answer

A: What do you see in this x-ray?
B: There appears to be a fracture of the right tibia.
A: That's right. Can you identify the type of fracture?
B: That looks like a standard stress fracture.
A: Are you sure? Maybe you should take a closer look.
B: Is that wrong? It's not compound. The bone isn't sticking out of the skin.
A: You're right, it's not compound. But can you see how the bone is slightly displaced?
B: Oh, yes, I see that now. So we'll need to realign the bone so that the two parts connect in the right place.

9 Suggested Answer

Location of injury/condition: Right arm
Description of injury/condition: The patient has a fracture of the right tibia. The bone appears to be displaced.
Steps needed for treatment: The bone must be re-aligned to ensure the parts connect in the correct place.

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Unit 3

1 Suggested Answers

1 The skin is the soft outer covering of the human body. It is made of three layers. The outermost layer of the skin is the epidermis.
2 Some skin conditions that are caused by external trauma include contusions and abrasions. Acne is a disease of the skin caused when oil becomes trapped in pores.

2 1 B 2 D 3 B

3 1 B 2 E 3 C 4 A 5 D

4
1 A subcutaneous fat B Acne
2 A epidermis B dermis

5 Suggested Answer

An abrasion is damage to the epidermus, the outermost layer of the skin.

6 1 T 2 F 3 F

7
1 acne
2 quite common
3 pimples
4 for you to do
5 soap
6 I'll give you

8 Suggested Answer

A: So Sara, what you have here is a common skin condition called acne.
B: Really? Is it bad?
A: It's not too bad. And it's quite common in teenagers like you.
B: Can we make it go away?
A: Possibly. There are a number of medicines we can try. But there are some things you can do as well.
B: Like what?
A: The best thing for you to do is to wash your face three times a day. Can you do that?
B: Yeah. Should I use special soap?
A: Yes. I'll give you some to take home.

9 Suggested Answer

Patient Name: Sara Andersen
Skin Condition: The patient has acne on her face and neck.
Treatment Options: I gave her one bottle of soap to treat acne.

Unit 4

1 Suggested Answers

1 The main parts of the nervous system are the brain, the spinal cord and nerves.
2 The nervous system controls all bodily actions - everything from running to breathing. Without this function we would be unable to respond to changes in our environment. A damaged nervous system can lead to permanent paralysis or death.

2 1 F 2 F 3 T

3 1 E 2 C 3 A 4 D 5 B

4
1 nervous system
2 vertebrae
3 nerve
4 neuron
5 network

5 Suggested Answer

Neurons are the basic building blocks of the nervous system. Everything else is built from them. Neurons connect to form networks, which process information. Information travels along nerves and up the spinal cord to the brain. The brain has billions of neurons that work together to control the body's systems.

6 1 A 2 B

7
1 types of signals
2 chemical or electrical
3 average brain
4 that's a tough one
5 It's actually
6 information exchange

8 Suggested Answer

A: What are the signals that synapses send?
B: They send chemical or electrical signals to other cells.
A: Very good. How many synapses are there in the body?
B: I would guess about 50 billion.
A: Actually, there are up to 500 trillion of them.
B: Gee! That is a lot of synapses.

9 Suggested Answer

City College of Medicine
Elizabeth Dorset

Nervous System Notes

What are synapses: They are the connections between nerve cells, or neurons.
Different kinds of synapses: There are two kinds of synapses. Some emit chemical signals, while others emit electrical signals.
Synapses in the human brain: There are up to 500 trillion synapses in the human brain.

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Unit 5

1 Suggested Answers

1 The endocrine system controls special chemicals that travel throughout the body to regulate certain functions. These functions include growth, hunger, thirst, sleep cycles, protein creation, and sexual functions.

2 There are many important glands in the endocrine system. These include the pituitary gland, the adrenal gland, the thyroid gland and the hypothalamus.

2 1 B 2 C 3 A

3 1 D 2 C 3 A 4 E 5 B

4

1	A	endocrine system	B	melatonin
2	A	thyroid gland	B	pineal body
3	A	adrenal gland	B	pituitary gland

5 Suggested Answer

There are many important glands in the endocrine system. The pituitary gland in the brain secretes hormones that control growth and several other functions. The thyroid gland in the neck controls how the body uses energy and makes proteins. The pineal body, in the middle of the brain, produces melatonin. This chemical helps regulate a person's sleep cycle.

6 1 F 2 F 3 T

7

1	are his symptoms	4	guess would be
2	extreme fatigue	5	cause of
3	sounds serious	6	enough hormones

8 Suggested Answer

A: Let's see if you can identify the patient's condition.
B: Okay, Doctor. What are his symptoms?
A: He has rapidly gained weight in his midsection. He sweats too much and bruises easily.
B: That sounds serious. Is there anything else?
A: He has recently lost quite a bit of hair.
B: That's interesting. Hmm ... based on those symptoms, my guess would be that the patient has Cushing's syndrome.
A: Well done! And what is the cause of this disease?
B: It's an endocrine system disorder. It occurs when the adrenal glands produce too much hormones. A tumor in the pituitary gland can cause it.

9 Suggested Answer

University Medical School
David Rice

Patient: Craig Stone

Symptoms: He gained weight rapidly. He sweats too much and bruises easily. He has also lost some of his hair.
Condition: Mr. Stone has Cushing's syndrome.
Cause: This disease occurs when the adrenal glands produce too many hormones. It is a rare, chronic endocrine system disorder.

Unit 6

1 Suggested Answers

1 There are a number of different parts to the reproductive system. Men have testes while women have ovaries.

2 It is important to practice safe sex for many reasons. Chief among these are preventing an unwanted pregnancy and stopping the transmission of STDs such as HIV. There are several methods available for birth control such as birth control pills and condoms.

2 1 F 2 T 3 F

3 1 B 3 F 5 D
2 C 4 A 6 E

4

1	A	reproductive system	B	gonads
2	A	sexually active	B	birth control
3	A	condom	B	genitalia

5 Suggested Answer

The clinic offers a variety of services to promote reproductive health. They counsel patients on the use of birth control and how to prevent STDs. They can help people choose and use different contraceptive methods. They also present treatment options to infertile couples.

6 1 D 2 D

7

1	sexually active	4	most of the time
2	use contraceptives	5	unwanted pregnancy
3	I try to	6	I just forget

8 Suggested Answer

A: I have a few questions before we get started with your physical exam.
B: Oh, sure. What do you need to know?
A: Let's see. First, are you sexually active?
B: Yes, I am.
A: And do you use contraceptives?
B: I do sometimes, but often I forget.
A: It's very important for you to use a contraceptive every time. If you don't, you could end up with an

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unwanted pregnancy. And you're putting yourself at risk for STDs. Do you have birth control pills?
B: Yes.
A: Make sure that you use them regularly. Also, your partner should always use a condom.

9 Suggested Answer

Southwest Reproductive Health Clinic
Using Contraceptives

Dangers of Not Using Contraceptives: It is very important to use contraceptives. Not using contraceptives could lead to an unwanted pregnancy or contracting a dangerous STD, such as HIV.
First Steps to Using Contraceptives: Many types of contraceptives are available. A condom should be used every time you have intercourse. We provide free condoms in our clinic.

Unit 7

1 Suggested Answers

1 The urinary system is vital to a body's overall health. It ensures that waste is removed promptly so only important materials are retained in the body.
2 There are many parts to the urinary system. These include the left and right kidneys, the abdominal aorta and the urinary bladder.

2 Symptoms: Patient experiencing restricted urine flow and discomfort of bladder for last four days.
Diagnosis: Patient has blocked urethra due to kidney stones.
Treatment: Recommended increasing fluid intake, and dietary changes.

3
1 kidney stones
2 bladder
3 urinary system
4 waste

4
1 casts
2 urethra
3 flow
4 urine
5 pass

5 Suggested Answer

Kidney stones can cause pain and poor urine flow due to blockage of the urethra.

6 1 T 2 F 3 F

7
1 kidney stone
2 so much pain
3 to pass through
4 getting stuck
5 too small
6 hurts so much

8 Suggested Answer

A: It looks like you're suffering from kidney stones.
B: Kidney stones? What are those?
A: A kidney stone is a collection of minerals in urine. Many people never notice them.
B: Then why am I hurting so much?
A: Because your urine has a high content of these minerals, the stones got too big to pass through your urethra.
B: So they're getting stuck?
A: That's right. The pain is your body trying to push the large stone through a space that is too small.
B: No wonder it hurts. What should I do?
A: Drink lots of water, and I'll prescribe something to ease the pain.

9 Suggested Answer

Patient: Richard Keel.
Condition: Patient has kidney stones causing trouble passing urine.
Is patient experiencing pain: Y / N
Treatment: Patient should drink lots of water and use painkillers if pain is severe.

Unit 8

1 Suggested Answers

1 A patient's medical history includes a range of information, including any complains they have and any illness they have a history of. A family medical history also records any conditions in the patient's family that might affect the patient's health.
2 Doctors use medical histories to gather information that might be relevant to a patient's current condition. The general information in the past medical history helps doctors establish whether a patient's current condition might be related to past problems.

2 Past Medical History: Previous medical problems, especially related to current condition
Social History: Patient's personal habits and lifestyle
Family Medical History: Conditions likely to occur in a particular family

3
1 C
2 A
3 D
4 F
5 E
6 B

4
1 A history of
2 A past medical history
B current
B social history

5 Suggested Answer

Medical professionals take medical histories to uncover information that might help with diagnoses.

6 1 D 2 A

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7 1 suffering from
2 on and off
3 six years ago
4 ever been treated
5 couldn't find
6 a history of

8 Suggested Answer

A: Your form says you're suffering from high blood pressure.
B: Yes. I've had this problem on and off for years.
A: Do you remember when exactly the onset of the problem was?
B: I first noticed it about five years ago.
A: Have you ever been treated for the condition?
B: Yes, I've received treatment before, but it didn't help.
A: Does anyone in your family have a history of hypertension?
B: I don't think so.

9 Suggested Answer

Patient: Max Smith
Current condition: High blood pressure
Onset: Started about five years ago
Treated previously? Y / N
If yes, explain: The patient has been treated, but treatments did not help condition.

Unit 9

1 Suggested Answers

1 Doctors can examine patients and perform tests to make diagnoses, but they also rely on patients' descriptions to help identify illnesses. Patients must be able to tell doctors where they feel pain or what bodily changes they experience. This gives doctors vital information that might not show up in an examination or lab test.
2 The flu is characterized by various uncomfortable symptoms. The patient usually experiences headaches and muscle aches, as well as general fatigue. Flu sufferers often have cold-like symptoms, such as coughing and a runny nose.

2 1 C 2 C 3 A

3 1 F 3 E 5 C
2 A 4 B 6 D

4 1 throbs 2 flu 3 fever 4 symptom

5 Suggested Answer

A patient with cold-like symptoms that are especially severe is likely to have the flu. A fever is a strong indication that a patient has the flu.

6 1 F 2 T 3 T

7 1 feel well
2 Have you been
3 hard to breathe
4 blowing my nose
5 muscle aches
6 high fever

8 Suggested Answer

A: What's the trouble today?
B: Well, Doctor, I think I have the flu.
A: Have you been coughing?
B: Mostly wheezing. It's hard to breathe.
A: Have you noticed that your nose is runny?
B: Yes. I've been blowing my nose constantly.
A: Do you have muscle aches?
B: Yes. Is it the flu?
A: Let me check your temperature. You do have a high fever. I think it is the flu. You'll need lots of rest and something to ease the symptoms.

9 Suggested Answer

Symptoms:
x fever
x muscle aches
x other; if yes, please describe: The patient is experiencing a runny nose, coughing, and wheezing.

Recommendations: The patient should get plenty of rest and take medication to relieve the symptoms.

Unit 10

1 Suggested Answers

1 Giving a physical examination involves checking the patient's vital signs and that all physical body parts are properly functioning. Examination techniques include inspection, and auscultation and percussion tests. A patient's body language is also checked.
2 Checking a patient's vital signs. It involves recording a patient's temperature, blood pressure, breathing, and pulse. Doctors check patient's breathing by using a stethoscope for auscultation. Pulse is checked by pressing on an artery in the patient's neck or arm or by using machines.

2 1 A 2 D 3 C

3 examination techniques - palpation, auscultation, percussion
things to check in an exam - pulse, body language, vital signs

4 1 inspection
2 observation
3 affect
4 evaluate

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5 Suggested Answer

The first step in conducting a physical exam is making sure that the patient is comfortable and that the examination environment is optimal.

6 1 F 2 T 3 F

7
1 arms at your sides
2 auscultation and percussion
3 swollen
4 next, I'm going to

8 Suggested Answer

A: Okay Dan. I'm going to ask that to sit up. I have to do an inspection of your lungs.
B: All right. Like this?
A: Yep. Just put your arms at your sides and relax.
B: Do you think something is wrong with my lung?
A: No. I just want to do some auscultation and tests to make sure. Take a deep breath.
B: (inhales)
A: I'm just trying to evaluate the quality of your respiration. Exhale.
B: (exhales)
A: Okay, next I'm going to tap your stomach.
B: Why do you do that?
A: It's a test to detect stomach problems. It's called percussion.

9 Suggested Answer

Patient: Dan Phillips
Physician: Dr. Carl Wayans
Date: June 12
Physical Examination
Auscultation: Auscultation of lungs found respiration normal.
Percussion: Percussion tests detected possible excess abdominal fluid.
Body Language: Patient's body language showed signs of pain.

Unit 11

1 Suggested Answers

1 There are many kinds of diagnostic tests such as a simple blood pressure check. Others require taking samples and processing them in the lab. These include a urinalysis for urine and BUN test for blood. More serious tests, like biopsies, can even involve surgical procedures.
2 Performing the right test on a patient is essential to determine the patient's problem or condition. Some tests may be very expensive or risky, so it is very important to know exactly what the patient needs. Administering the wrong test could cause complications and will likely require further tests.

2 1 F 2 F 3 T

3
1 C 3 D 5 B
2 F 4 E 6 A

4
1 renal
2 biopsy
3 blood pressure
4 kidney
5 pathology report
6 GFR test

5 Suggested Answer

All of the patient's test results point to serious kidney disease. His blood pressure is too high. There are very high levels of albumin and creatinine in the patient's urine and blood, respectively.

6 1 D 2 C

7
1 test results back
2 results are serious
3 kidney disease
4 not severe
5 BUN test
6 dangerous combination

8 Suggested Answer

A: Hello, Mr. Reed. I have your test results back.
B: How do they look?
A: Well, the results are serious. First, according to the GFR test, your GFR level is 38.
B: So is that too low?
A: Yes. It indicates a moderate case of kidney disease.
B: Well, at least it's not severe I suppose.
A: No, but it could become serious. Also your BUN level is rather high. That could be a dangerous combination with a low GFR rating.
B: I see. So what do we do?
A: We need to perform a renal biopsy as soon as possible.

9 Suggested Answer

Heartland Hospital
Lab Test Results
Patient: Clark Reed

GFR test: Mr. Reed GFR level is 38. This is seriously low.
BUN test: The BUN test showed an abnormally high BUN level.
Diagnosis: The combination of a low GFR rating and high BUN level indicated that Mr. Reed has a moderate case of kidney disease. The disease is in danger of rapidly becoming worse.
Further Testing: A renal biopsy needs to be performed as soon as possible.

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Unit 12

1 Suggested Answers

1 There are many types of medical imaging. X-rays use small amounts of radiation to take pictures of the inside of the body. A doctor may use an endoscope - a tubular instrument with a camera on the end of it - to look inside the body. Other types include ultrasound imaging and MRI and CT scans.

2 Health professionals must know exactly what kind of medical imaging the patient and situation need. Using the wrong kind of imaging may miss the problem or lead to an incorrect diagnosis. In addition, some medical imaging techniques, such as an MRI, can be very expensive.

2 1 C 2 B 3 C

3 1 B 2 A 3 D 4 E 5 C

4
1 ECG
2 gel
3 feed
4 X-ray
5 medical imaging

5 Suggested Answer

The clinic offers several imaging services. X-rays and CT scans use radiography to take pictures, while ultrasounds use the reflection of sound waves. Tiny cameras at the end of endoscopes allow doctors to view inside the body during exams or surgery. An MRI uses magnetic properties of atoms to create very detailed pictures.

6 1 T 2 T 3 F

7
1 taking an X-ray
2 that doesn't work
3 Have you ever had
4 Could you explain
5 radiography
6 three-dimensional image

8 Suggested Answer

A: Suzanne, we're going to start off by taking an X-ray of your brain. That might show the cause of your headaches.
B: Okay, doctor. What if that doesn't work?
A: We might need to do a CT scan. Have you had one before?
B: No, I haven't. Could you explain it to me?
A: Sure. It's a type of radiography, but using many different X-rays.
B: I see. Anything else?
A: Yes. Then a computer combines the X-rays into a three-dimensional image, and we work with the image to see many different things.

9 Suggested Answer

Community Health Clinic
CT Scan Information

When it is needed: Sometimes a basic X-ray does not tell us exactly what we need to know. In those cases, a CT scan may be called for.
How it works: A CT scan takes many different two dimensional X-rays. A computer can then combine the X-rays into one three-dimensional image.

Unit 13

1 Suggested Answers

1 In my country, most family medicine practitioners work in clinics, though some may also work in hospitals. They serve as primary care practitioners for many different kinds of patients, regardless of their age, gender, or medical condition. They are the first doctor a patient will see if they have a non emergency medical problem.

2 Family practitioners treat patients' injuries and illnesses through curative medicine. They also practice preventative medicine through health education and promotion of good health habits. If needed, many family practitioners can provide urgent care services to patients requiring immediate attention.

2 1 B 2 C 3 A

3 1 D 2 C 3 A 4 E 5 B

4
1 special practitioner
2 advise
3 refer
4 counsel

5 Suggested Answer

Family practitioners treat patients' injuries and illnesses through curative medicine. They also practice preventative medicine through health education and promotion of good health habits. They may also need to refer a patient to another doctor to address their specific medical needs. If needed, many family practitioners can provide urgent care services to patients requiring immediate attention.

6 1 F 2 T 3 T

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7

1	You'll want to	4	refer you to
2	If it still hurts	5	make an appointment
3	physical therapy	6	as soon as

8 Suggested Answer

A: Here's what you have to do. For a couple of days, place a ice pack on it for ten to twenty minutes every hour.
B: I can do that. What else? Should I see an eye doctor?
A: Yes. I'm going to refer you to Dr. McDonald. She'll make sure you don't have any long-term eye problems.
B: When should I make an appointment with her?
A: Anytime that fits in your schedules is fine.
B: Sounds great. Thank you very much, Doctor.

9 Suggested Answer

Family Health Clinic
Home Care Instructions

Patient: Brian Jordan

Home Care: Put an ice pack on the injury for ten to twenty minutes every hour.
Specialist Referral: Make an appointment with Dr. McDonald whenever suits you.

Unit 14

1 Suggested Answers

1 Pediatrics is medical care for children. This includes care for newborn infants, young children, and teenagers.
2 Some illnesses are commonly found in infants and young children. These include jaundice, asthma, and obesity.

2 1 F 2 F 3 F

3 Age Categories - infant, newborn, teenager
Health Problems - asthma, jaundice, obesity

4

1	A bilirubin	B	Juvenile
2	A check ups	B	pediatrics

5 Suggested Answer

Childhood obesity is an illness wherein a child develops an unhealthy amount of excess body fat. Treatment of obesity includes diet programs.

6 1 B 2 A

7

1	appointments	4	jaundice
2	so far	5	billi light
3	coming in		

8 Suggested Answer

A: Hi, Sam. How many appointments do we have for this morning?
B: Hello, Doctor. We have two appointments set up so far.
A: Okay. When is the first one?
B: The first appointment is at nine-thirty. Gina Simms is coming in for her asthma treatments.
A: Good. When is the next one?
B: Then you have an infant at ten. She is showing signs of jaundice.
A: Oh that's right. I'll have to get the bili light ready. Can we push the appointment back to ten-thirty?
B: Sure. I'll give the mother a call now.
A: Great. Thanks, Sam
B: Not a problem, Doctor.

9 Suggested Answer

Doctor Harding,

This morning's schedule of appointments is as follows.

Time	Patient	Age	Condition
9:30	Gina Simms	Teenager	Asthma
10:00	Carol Oates	Infant	Jaundice

Unit 15

1 Suggested Answers

1 There are many diseases that are more likely to affect older patients. This is because of their advanced age and weakened immune system. Seniors are at increased risk for a stroke and hearing loss. Seniors who cannot move much or are not taken care of may develop bed sores.
2 Senior citizens often require specialized care. They may need to take several medications at once and need help understanding when and how much of these drugs to take in order to avoid polypharmacy. Seriously ill seniors may need to be temporarily or permanently placed in a nursing home.

2 1 F 2 T 3 F

3

1	C	3	F	5	A
2	D	4	B	6	E

4

1	drug interaction	4	elderly
2	Alzheimer's disease	5	dizziness
3	assisted living	6	hearing loss

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5 Suggested Answer

Elderly patients often require very specialized care. They can develop serious diseases not usually seen in other age groups, such as Alzheimer's. They must often take several medications. They may need help understanding how to take these drugs to avoid dangerous interactions. Seniors who require help may be placed in assisted living or a nursing home, depending on the seriousness of their condition.

6 1 C 2 D

7

1	read the label	4	That could cause
2	What medication	5	I'll make sure
3	heart rhythm	6	standard care

8 Suggested Answer

A: Ms. Johnson has recently started a new medication. You might have to read the label and make sure she understands it.
B: What medication is she taking?
A: She's on blood pressure medicine right now. While she's taking that, she can't have her other meds. It could cause a dangerous interaction.
B: Okay, I'll make sure she knows to stay off the other drugs.
A: Good. The rest of the patients require standard care.
B: That sounds fine. I'm ready to start rounds.

9 Suggested Answer

Assisted Living Center
Nurse: James Cobb
Wing: 2 East

Patient: Clare Johnson
Special Care Required: Ms. Johnson is taking medication for blood pressure. I must make sure she doesn't take her other meds.

Patient: Beth Clark
Special Care Required: Ms. Clark has started taking Cordarone for an abnormal heart rhythm. I need to make sure she knows when and how to take it. I also need to make sure she is not taking Zocor. Taking both could cause a dangerous interaction.

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Unit 1

Nurse (F): Okay, Mr. Lewis, we're almost done taking your blood. Hold still for a little longer.
Donor (M): Sure. So, where will my blood go?
Nurse: Our facility provides blood to local hospitals and surgical centers for transfusions.
Donor: I see. So anyone might receive my blood.
Nurse: Not quite. Your blood is Type A, which means that it's only compatible with other people who have Types A and AB. We can't give your blood to someone with Type O or B.
Donor: Really? What happens if someone gets a transfusion with the wrong type?
Nurse: That can be very dangerous, or even fatal.
Donor: Wow. So you have to be pretty careful about labeling, don't you?
Nurse: That's right. We keep very careful records here to make sure no one gets the wrong blood type.

Unit 2

Doctor (F): So, Roger, tell me what you see in this x-ray.
Student (M): Hmm, let's see. Well, there appears to be a fracture of the left tibia.
Doctor: That's right. Can you identify the type of fracture?
Student: It doesn't look too serious. I'd say that's a pretty standard stress fracture.
Doctor: Are you sure? Maybe you should take a closer look.
Student: Is that wrong? It's not comminuted, is it? I don't see multiple bone fragments.
Doctor: No, it's not comminuted. You're right about that.
Student: Then I don't know. What am I missing?
Doctor: Can you see how the bone is slightly displaced?
Student: Where? Oh, I see that now. Of course!
Doctor: So what do we have to do before the bone starts healing?
Student: We'll need to realign the bone so that the two parts connect in the right place.

Unit 3

Doctor (F): So James, it looks like you've developed acne. Have you heard of it before?
Patient (M): Yeah, but I don't know much about it. Is it bad?
Doctor: It's not too bad. It's actually quite common in teenagers.
Patient: What causes it?
Doctor: Oils that become trapped in the skin's pores cause it. This is why the pimples form.
Patient: Can we make it go away?
Doctor: The best thing for you to do is to wash your face twice a day. Can you do that?
Patient: Yeah. Should I use special soap?
Doctor: Yes. I'll give you some to take home.
Patient: Is that all?
Doctor: Well, we can start with some topical medicine. If that doesn't work I'll give you a prescription for some pills.

Unit 4

Professor (M): Class, let's see if you've been studying the nervous system. Elizabeth, would you answer a few questions for me?
Student (F): Certainly, Professor.
Professor: First, what are the connections between neurons called?
Student: Those are synapses, Professor. They enable the transfer of information.
Professor: Very good. And what types of signals do synapses send?
Student: They send chemical or electrical signals to another cell.
Professor: Right again. So tell me, about how many synapses are there in the average brain?
Student: Hmm... that's a tough one. I would guess about 100 billion.
Professor: Not quite. It's actually many times that - 100 to 500 trillion.
Student: Wow, that's a lot of synapses.
Professor: Indeed. All that information exchange is what makes the brain so powerful.

Unit 5

Doctor (F): David, let's see if you can identify Mr. Stone's condition.
Student (M): Okay, Doctor. What are his symptoms?
Doctor: He has extreme fatigue and feels light-headed. He has lost weight and suffers from diarrhea and nausea.
Student: That sounds serious. Is there anything else?
Doctor: He's lately had a strong craving for salty foods.
Student: That's interesting. Hmm... based on those symptoms, my guess would be that Mr. Stone has Addison's disease.
Doctor: Well done. And what is the cause of this disease?
Student: It's a rare endocrine system disorder. It occurs when the adrenal glands don't produce enough hormones.
Doctor: Very good. How is this disease treated?
Student: Since it's a chronic disease, treatment will be lifelong. It involves regular steroid replacement therapy.
Doctor: Right again. You're doing well, David. Let's move on to the next case.

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Audioscripts

Unit 6

Doctor (M): Okay, Ms. Ryan. I have a few questions before we get started with your physical exam.
Patient (F): Oh, sure. What do you need to know?
Doctor: Let's see. First, are you sexually active?
Patient: Yes, I am.
Doctor: And do you use contraceptives?
Patient: Um, well I try to.
Doctor: Try to?
Patient: Well, I use them most of the time.
Doctor: Ms. Ryan, it's very important for you to use a contraceptive every time. If you don't, you could end up with an unwanted pregnancy. And you're putting yourself at risk for STDs.
Patient: I know. I guess sometimes I just forget.
Doctor: I see. So you forget your birth control pill, or you forget to use a condom?
Patient: Condoms. I don't take birth control pills.
Doctor: Well, you should use one every time you have intercourse. But I'm going to recommend another form of birth control, too.

Unit 7

Doctor (F): Well, James, It looks like you're suffering from kidney stones.
Patient (M): Kidney stones? What are those?
Doctor: A kidney stone is a small collection of minerals that forms in urine. Many people get them and never even notice.
Patient: Then why am I in so much pain?
Doctor: Well, here's what happened. Since your urine has such a high content of these minerals, the stones got too big to pass through your urethra.
Patient: Oh, I see. So they're getting stuck?
Doctor: That's right. The pain you feel is your body trying to push the large stone through a space that is too small.
Patient: No wonder it hurts so much. What should I do?
Doctor: Drink lots of water. I know it hurts, but we need to get your body to expel those stones. And I'll prescribe something to help ease the pain.

Unit 8

Nurse (M): The doctor will be in shortly, Ms. Granger. While you're waiting, I have a few questions about your past medical history.
Patient (F): Of course. What do you need to know?
Nurse: You wrote on the form that you're suffering from chest pain.
Patient: Yes. I've had this problem on and off for years.
Nurse: Do you remember when exactly the onset of the problem was?
Patient: Let's see. I think I first noticed it about six years ago.
Nurse: Have you ever been treated for the condition?
Patient: No. I did see a doctor once about it, but he couldn't find the cause.
Nurse: Does anyone in your family have a history of heart problems?
Patient: No, not that I know of. Do you think that could be the problem?
Nurse: We don't know anything yet. We just want to get as much information as we can so the doctor can make an informed diagnosis.

Unit 9

Doctor (M): What's the trouble today?
Patient (F): Well, Doctor, I don't feel well at all. I'm sure I have the flu.
Doctor: I'm sorry to hear that. Have you been coughing?
Patient: A little. But mostly I'm wheezing. It's hard to breathe.
Doctor: And have you noticed that your nose is runny?
Patient: Oh, yes. I've been blowing my nose constantly.
Doctor: Okay. Do you have pain throughout your body?
Patient: I do have muscle aches. And I'm so tired that I can hardly do anything. Do you think it's the flu, Doctor?
Doctor: Well, you have a pretty high fever. It looks like you're right about the flu. You'll need plenty of rest, and I'll prescribe something to ease the symptoms.

Unit 10

Doctor (M): Okay Sheryl, I'm going to ask that you lie down on the table. I have to do an inspection of your abdomen.
Patient (F): All right. Like this?
Doctor: Yes. Just put your arms at your sides and relax.
Patient: Do you think something is wrong with my stomach?
Doctor: No. I just want to do some auscultation and percussion tests to make sure. Take a deep breath.
Patient: (inhales)
Doctor: I'm just checking that nothing feels swollen. Exhale.
Patient: (exhales)
Doctor: Okay, next I'm going to tap right here on your stomach.
Patient: Why do you do that?
Doctor: This is a test to detect excess abdominal fluid.
Patient: Is there any?
Doctor: Nope. It all looks normal.

Unit 11

Nurse (F): Hello, Mr. Reed. I have your kidney test results back.
Patient (M): How do they look?
Nurse: Unfortunately, the results are serious. First, according to the GFR test, your GFR level is 40.

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Patient: So is that too low?
Nurse: Yes, it indicates a moderate case of kidney disease.
Patient: Well, at least it's not severe I suppose.
Nurse: No, but it could be headed that way. Also, according to the BUN test, your BUN level is rather high. That could be a dangerous combination with a low GFR rating.
Patient: I see. So what do we do?
Nurse: We need to perform a renal biopsy as soon as possible.
Patient: Okay. When should I come in for that?
Nurse: Dr. Roth has cleared some time for you two days from now.
Patient: That's fine. Thanks for all your help.

Unit 12

Doctor (M): Suzanne, we're going to start off by taking an X-ray of your brain. Maybe that will show the cause of your headaches.
Patient (F): Okay, Doctor. What if that doesn't work?
Doctor: Then we might need to do a CT scan. Have you ever had one of those before?
Patient: No, I haven't. Could you explain it to me?
Doctor: Sure. It's a type of radiography, but we take many different X-rays.
Patient: I see. Is that it?
Doctor: No. Then a computer combines the X-rays into a three-dimensional image. We can work with the image to see many different things.
Patient: That sounds really powerful.
Doctor: It is. It's expensive, though, so we'll start with the X-ray and see what that tells us.
Patient: How long will it take to get the X-ray?
Doctor: Radiology should have it for us in about an hour.
Patient: Okay. Thank you, Doctor.

Unit 13

Doctor (F): Okay, Brian, you're all set to go home now.
Patient (M): Thanks for taking care of me, Doctor.
Doctor: No problem. But before you go, I need to tell you how to take care of that broken arm.
Patient: Okay. I hope it's not too hard.
Doctor: Not really. Here's what you have to do. You'll want to elevate the arm to ease the swelling and pain.
Patient: I can do that. What else?
Doctor: If it still hurts, you can put an ice pack on it for a few minutes at a time. Put a towel in between the pack and your cast.
Patient: That's easy enough. Will I need physical therapy?
Doctor: Yes. I'm going to refer you to Dr. Rossman. He specializes in therapy after broken bones.
Patient: When should I make an appointment with him?
Doctor: You can start working with him as soon as your cast comes off.
Patient: Sounds great. Thank you very much, Doctor.

Unit 14

Doctor (M): Hi, Beth. How many appointments do we have for this morning?
Receptionist (F): Hello Doctor. We have three appointments set up so far.
Doctor: Great. When is the first one?
Receptionist: The first appointment is at eight. Michael Taylor is coming in for his asthma treatments.
Doctor: Good. When is the next one?
Receptionist: Then you have a three-day-old infant at nine. She is still showing signs of jaundice.
Doctor: Oh that's right. I'll have to get the bili light ready. Is there another appointment after her?
Receptionist: Yes. You have a teenager with obesity. He is coming in for a check up, but not until eleven.
Doctor: Okay. Can we push the nine o'clock back to nine-thirty.
Receptionist: Sure. I'll give the mother a call now.
Doctor: Great. And let me know if you set up anything else.
Receptionist: I will Doctor.
Doctor: Thanks Beth.

Unit 15

Nurse 1 (M): Sherri, let's talk about a couple of the patients in this wing before we start rounds.
Nurse 2 (F): Sure. What do I need to know?
Nurse 1: First, Mr. Bowman hasn't been too mobile recently. You might have to help him around a bit.
Nurse 2: Okay, that's no problem.
Nurse 1: Also do a quick check for bed sores.
Nurse 2: Sure thing. Who's next?
Nurse 1: Ms. Clark has recently started a new medication. You might have to read the label and make sure she understands it.
Nurse 2: What medication is she taking?
Nurse 1: She's on Cordarone for an irregular heart rhythm. While she's taking that, she can't have any Zocor for her cholesterol. That could cause a dangerous interaction.
Nurse 2: Okay, I'll make sure she knows to stay off the Zocor.
Nurse 1: Good. The rest of the patients require standard care.
Nurse 2: That sounds fine. Let's get started.

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Unit 1

1 Suggested Answers

1 Medical students learn a subject that is vital to the well being of their communities. As future doctors, they must be prepared to protect the health of others. Medicine is a complicated subject, so doctors must have a solid knowledge of how the body works in order to treat a wide variety of medical conditions.

2 In my country, a medical student must first complete a four-year bachelor's degree, before going to medical school. Most students earn a medical degree after another four years and then undergo a residency to get advanced training in a particular field. Finally, he or she must pass a Medical Licensing Examination to become a licensed doctor.

2 Before Medical School: premedical studies, entrance examination
After Medical School: residency, fellowship, foundation program
Licensing: test, fee, background check

3 1 B 3 E 5 F
2 A 4 C 6 D

4 1 A undergraduate B fellowship
2 A bachelor's degree B medical degree

5 Suggested Answer

A residency gives a new doctor the opportunity to receive specialized training in a hospital alongside experienced doctors.

6 1 T 2 T 3 F

7
1 let's talk about
2 well qualified
3 it says here
4 top of my class
5 best and brightest
6 orthopedic surgery
7 past eight months

8 Suggested Answer

A: It says here you attended Wayne Michaels Medical School.
B: That's right. I graduated at the top of my class.
A: That's excellent. What about your residency?
B: I completed my neurology residency at Jackson Hospital.
A: That's what we're looking for. How long have you been licensed?
B: I passed the medical exam last year, and I've been practicing for the past ten months.
A: So you don't have a lot of experience in actual practice.
B: The program at Jackson gave me lots of real-world experience.

9 Suggested Answer

Applicant's education: The applicant graduated at the top of his class at Wayne Michaels Medical School.

Applicant's experience: The applicant completed a residency at Jackson Hospital and has been in practice for ten months.

I plan to hire the applicant because he has an excellent educational background, even though his experience is limited.

Unit 2

1 Suggested Answers

1 Patient health relies on good communication between medical professionals. A patient who stays in a hospital is likely to be under the care of several doctors and nurses. Good communication keeps the patient safe and healthy.

2 Doctors must leave accurate records to ensure good communication. If the patient is undergoing treatment, their medical record should include a schedule of treatments or medications in chronological order. All notes and instructions must be concise and legible to avoid confusion.

2 1 F 2 T 3 F

3 1 A 3 B 5 D 7 F
2 C 4 H 6 E 8 G

4 1 chronological order 3 legible
2 shift change

5 Suggested Answer

Poor communication can lead to errors in patient care. A patient might receive the wrong medication, or receive too much or too little of it.

6 1 B 2 D

7
1 due for his
2 medical record
3 can't read
4 he's had any
5 can't be right
6 chronological order
7 a mess

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8 Suggested Answer

A: Is this patient due for his medication?
B: What does his medical record say?
A: I can't read these notes.
B: Let me see ... This doesn't make any sense. I think he's supposed to get his meds every day at eleven AM, but it doesn't look like he's had any dosages for the last few days.
A: That can't be right.
B: Wait, I think I see what happened. These notes aren't in chronological order. Who was on duty this morning?
A: I'll check the shift board ... Al was here this morning.
B: We'd better call him.
A: That's a good idea. We certainly don't want to give the patient the wrong dosage.

9 Suggested Answer

To: Hospital Director

We had a problem with a patient's medical records today. The notes about the patient's medication were not in chronological order. We were not able to figure out if he had received his medication today, so we contacted the nurse on duty this morning and got the necessary information.

Unit 3

1 Suggested Answers

1 There are many possible causes of patient miscommunication. These include poor health literacy, anxiety and cultural differences.
2 There are several easy ways to improve communication with patients. Above all, it is vital to establish a friendly rapport with the patient. Show empathy to your patients and demonstrate that you care about them and their problems.

2 1 B 2 A 3 D

3 1 E 3 C 5 B
2 F 4 A 6 D

4
1 A jargon B empathy
2 A miscommunication B health literacy
3 A cultural differences B preconceptions

5 Suggested Answer

It is important for medical professionals to help patients relax because if a patient feels comfortable, he or she is more likely to share more information with the professional.

6 1 F 2 T 3 F

7
1 just be anxious
2 pretty nervous
3 lots of jargon
4 nausea and anxiety
5 not mean much
6 make sure to

8 Suggested Answer

B: I can't tell. He's barely speaking to me.
A: Does he have a cultural background that might make him nervous in a hospital?
B: That could be. I'll try to be more sensitive about his beliefs.
A: Also, did you use lots of jargon? In my experience, that makes things worse.
B: I might have. I noted that his blood pressure was high, so I asked him if he had a history of hypertension.
A: See, that term may not mean much to him. Just say high blood pressure instead.
B: Good point.
A: Also, make sure to establish a rapport. Even just by asking about his day.
B: That's a good idea. Maybe it will make him feel more comfortable.

9 Suggested Answer

Heartland Hospital
Patient Communication Guidelines

Reducing anxiety: Many patients come to the hospital feeling very nervous. This is natural. It can be very scary to feel sick and not understand why. Consider this as you talk with the patient.
Avoiding jargon: Medicine is full of technical vocabulary. It can be easy to forget that many people do not know what terms mean. Explain complex concepts in simple ways that anyone can understand.
Building a rapport: Some patients want to immediately discuss their problems with you. But many others may want to chat for a bit. Engage them in friendly conversation. This will make them feel more comfortable.

Unit 4

1 Suggested Answers

1 Hospitals suffer from economic downturns just like any other business. The effects can be particularly severe since people's lives often depend on a hospital's care.
2 In difficult economic times doctors have to take on larger caseloads and paperwork. This can lead to exhaustion. They might also find it hard to pay back any loans they have.

2 1 D 2 A 3 A

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3 1 malpractice
2 paperwork
3 budget
4 debt

4 1 A shortage B caseload
2 A demand B loan
3 A exhaustion B shift

5 Suggested Answer

A hospital might need new doctors, but might not be able to hire them because it doesn't have enough money in the budget.

6 1 F 2 T 3 F

7 1 an hour ago
2 way behind
3 my day off
4 good for us
5 hospital budget
6 take a vacation

8 Suggested Answer

A: What are you doing here? I thought your shift ended already.
B: I have to finish this paperwork. I'm way behind.
A: I came in on my day off last week to finish mine. I asked the director for another doctor, but she said there's not enough money in the budget.
B: The whole situation is terrible. I want to take a vacation, but I need the money to pay off my student loans.

9 Suggested Answer

I worked an extra six hours this week to complete paperwork for patient files. Because of increases in my caseload, I was not able to complete this paperwork during my normal shift, so I stayed late and came in on my day off to complete it.

Unit 5

1 Suggested Answers

1 A pain that is not strong might described as mild or moderate. A very strong pain might be described as being excruciating or burning. This type of pain is frequently a constant or sharp pain.
2 Understanding how much pain a patient is feeling can help doctors make the right diagnosis as well as improve the patient's condition. For example, whether a pain is constant or intermittent can suggest different ailments. And if a patient is experiencing more than mild distress, a doctor will know to provide medication or treatment to lessen the pain as soon as possible.

2 1 F 2 T 3 F

3 1 intermittent
2 shooting
3 unbearable
4 constant

4 1 A distress B intensity
2 A mild B excruciating
3 A burning B sharp
4 A moderate B throbbing

5 Suggested Answer

If a patient's pain comes and goes, they can describe it as intermittent. If a patient always feels pain, it is constant.

6 1 B 2 D

7 1 of the pain
2 at worst
3 cut back
4 cause
5 throbbing
6 goes through

8 Suggested Answer

B: No, I'm having neck problems now. The pain is constant.
A: On a scale of one to ten, how would you rate the intensity of the pain right now?
B: Right now, it's mild. Like a two.
A: And how does it feel at worst?
B: At worst it's unbearable. Especially while I'm at work. I'm a truck driver.
A: I see. Perhaps that's contributing to your neck pain. Now, can you describe how it feels? Does it burn or throb?
B: Mostly the pain is burning. Occasionally I get a sharp pain at the base of my neck.
A: Does it shoot down your spine?
B: Sometimes. Not usually.

9 Suggested Answer

Patient Assessment Form

Patient Name: Sam Rodriguez
Age: 30
Location of pain: The patient says that he gets neck pain.
Intensity of the pain: The patient says that his pain is mild at best, and unbearable at worst. He feels it the most when he's at work.
Patient's description of the pain: The patient says that his neck pain is constant. He typically gets it while working. He says the pain is burning and he occasionally gets a sharp pain at the base of his neck.

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Unit 6

1 Suggested Answers

1 There are many kind of chronic and acute illnesses. Pneumonia, a dangerous lung condition, is an acute illness. Chronic bronchitis is a common chronic illness.
2 Even a disease that does not seem serious can eventually cause complications. For this reason it is important to always have a disease evaluated by a health professional.

2 1 B 2 A 3 D

3 1 E 3 B 5 A
2 D 4 F 6 C

4
1 pre-existing
2 conditions
3 chronic
4 intervention
5 acute

5 Suggested Answer

Chronic conditions that are left untreated can develop into serious medical problems. They will likely not go away on their own but rather will grow worse over time. They may also trigger the onset of a dangerous acute condition.

6 1 T 2 F 3 T

7
1 Do you know what
2 have no idea
3 Can we treat
4 quite some time
5 caused by the strep virus
6 knock out
7 life complications

8 Suggested Answer

A: You've come down with a bad case of pneumonia. Do you know what that is?
B: Yes.
A: Okay. I'll start you on some antibiotics immediately. But this could have been avoided.
B: Really? How so?
A: Well, you've been sick for quite some time, haven't you?
B: Yeah, but I figured it was just a cold.
A: Definitely not. Normally, we can knock out cases of pneumonia pretty quickly. But untreated, the infection gets worse. Now we have to get rid of that infection before it becomes a lifelong condition.

9 Suggested Answer

Patient Record

Patient: Jack Anderson
Symptoms: Mr. Anderson has several symptoms similar to a cold.
Acute Conditions: Mr. Anderson has pneumonia. Mr. Anderson did not seek treatment for several days. In that time, his infection has gotten worse.
Chronic Conditions: Mr. Anderson may or may not develop chronic pneumonia. We are starting him on antibiotics immediately to avoid a chronic condition.

Unit 7

1 Suggested Answers

1 There are three types of diabetes. There are type 1 and type 2 and gestational diabetes. All types of the condition reduce the body's ability to regulate its amount of blood sugar.
2 There are many risk factors associated with diabetes. These include blindness through cataracts, gum disease and amputations.

2 1 D 2 C 3 B

3 1 D 3 F 5 E
2 A 4 C 6 B

4
1 type 1 diabetes
2 gum
3 blindness
4 type 2 diabetes
5 BMI
6 gestational diabetes

5 Suggested Answer

Effective diabetes management is critical to maintaining long-term health in a diabetic patient. Lifestyle changes such as increasing exercise or reducing fat and alcohol consumption are often a part of this. Also, many diabetic patients take insulin medication to regulate their blood sugar levels.

6 1 F 2 T 3 T

7
1 insulin shots
2 We'll start with
3 sugar-free candy
4 meal plan
5 Will I have to
6 Not necessarily

8 Suggested Answer

A: Hmm. To be honest you have a pretty high BMI. That's a big risk factor for developing diabetes.
B: I see. Will I need to really restrict my diet then?
A: Not necessarily. We'll start with a lifestyle plan that includes normal healthy eating guidelines.
B: So I'll have to give up chocolate, pasta, and all sorts of stuff?
A: Actually, no. You will probably have to eat less than you do now. But small amounts can still be okay.
B: That's good. Will I have to lose a lot of weight?
A: Not a lot.

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9 Suggested Answer

Family Health Clinic
Diabetes Treatment Plan

Risk Factors: There are many risk factors for diabetes. Some, like genetics and race, are beyond the patient's control. Others, like a diet high in fat or alcohol, can and should be managed both before and after diagnosis.
Diet: Contrary to popular belief, diabetic patients are not limited to a very strict diet. They should control the amount of sweets, starch, fats, and other foods they eat. This is the same that would be recommended for any healthy diet.
Exercise: Again, diabetic patients are not required to follow very strict rules about exercise. They should exercise regularly just as any healthy person. Proper lifestyle management can reduce or eliminate the need for regular insulin injections.

Unit 8

1 Suggested Answers

1 The most common type of cancer is prostate cancer. However, by far the deadliest form of cancer in my country is lung cancer. Another very common form of cancer is breast cancer in women.
2 There are several forms of treatment available to cancer patients today. These include radiation therapy, photodynamic therapy and chemotherapy.

2 1 T 2 F 3 T

3 1 C 2 E 3 D 4 A 5 B

4
1 photodynamic therapy
2 cancer
3 oncologist
4 outpatient

5 Suggested Answer

Patients are given a light sensitive drug that contains cancer destroying substances. Light beams then activate the drug.

6 1 C 2 D

7
1 all ears
2 regular doses
3 side effects
4 feel nauseous
5 another option
6 photodynamic therapy
7 a light beam

8 Suggested Answer

B: I'm all ears.
A: First, there's radiation therapy. We use powerful X-rays to kill the cancer cells.
B: I don't know how I feel about that. Are there side effects?
A: There can be. You'll likely feel fatigued and there will be some skin irritation in the areas that we focus the X-rays.
B: I see. Is there another option available?
A: We may be able to try immunotherapy.
B: I've never heard of that.
A: We make some antibodies in the lab. Then we introduce them into your body to strengthen your immune system. They target the cancer cells and kill them.
B: That sounds interesting. What would you recommend?
A: We could try both. It would limit the amount of radiation you're exposed to.

9 Suggested Answer

Family Health Clinic
Cancer Treatment Options

Chemotherapy: Chemotherapy is a very common cancer treatment. It uses regular doses of strong drugs to kill cancer cells. These drugs can be taken orally, but are usually given through an IV injection.
Side Effects: Because chemotherapy drugs are so strong, there are some potentially serious side effects. Most patients will feel nauseous and lose most of their hair. Specific organs such as the liver or heart can also be damaged.
Photodynamic therapy: Photodynamic therapy is a relatively new form of cancer treatment. The patient is given a cancer-killing drug that is sensitive to light and absorbed by the tumor. Then light beams are focused on the tumor, activating the drug and killing cancer cells.

Unit 9

1 Suggested Answers

1 There are several kinds of heart disease. In my country, heart disease is the leading cause of death. Two common forms are coronary artery disease and atherosclerosis.
2 There are many simple steps that everyone can follow to lower their risk of getting heart disease. Smokers should seriously consider quitting. Getting regular exercise, eating a healthy diet and reducing stress will also help.

2 1 F 2 T 3 F

3 1 E 2 A 3 B 4 C 5 D

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4 1 harden
2 Smoking
3 bypass
4 stent
5 cholesterol

5 Suggested Answer

There are a few different approaches to treating heart disease. Many patients with heart disease will take regular medications. There are also surgical options available, like angioplasty and bypass surgery.

6 1 A 2 B

7 1 coronary artery disease
2 might just be
3 just from my
4 warning signs
5 looking at
6 It depends on
7 could lead to

8 Suggested Answer

A: It could be arrhythmia or possibly atherosclerosis.
B: Shouldn't we be able to tell what I have, just from my symptoms?
A: Not necessarily. Rapid heart rate changes are symptoms of arrhythmias and atherosclerosis.
B: So what types of treatment am I looking at?
A: It depends on your diagnosis. If it's arrhythmia, we might not need to do anything.
B: Really?
A: Yes, depending on how serious it is. Otherwise some medication may be necessary.
B: I see. What if it's atherosclerosis?
A: That could be treated with medications called statins and lifestyle changes. Surgery may also be necessary.
B: Okay. I hope it's just an arrhythmia.
A: We have to run some tests to find out.

9 Suggested Answer

Family Health Clinic
Heart Condition Information
Coronary Artery Disease: This is a serious heart condition in which the arteries of the heart become blocked. This restricts blood flow to the heart.
Symptoms: Shortness of breath, chest pain, and irregular heart beats.
Treatment: CAD can be treated in several different ways. Regular medication along with changes to diet and exercise are usually the first steps. Should these fail to improve the condition, surgery may be necessary. Angioplasty can widen narrow arteries, while open heart bypass surgery reroutes blood flow around blocked arteries.

Unit 10

1 Suggested Answers

1 An emergency room must be prepared to see all kinds of patients. The worst patients are with trauma. Other patients may come to the ER for less serious reasons, such as routine illnesses or minor injuries.
2 Patients go to the triage station. There staff prioritize the patients so that those patients most in need get attention first.

2 1 F 2 T 3 F

3 1 C 2 G 3 E 4 A 5 B 6 F 7 D

4 1 acuity
2 Emergency Severity Index
3 injury
4 triage station
5 trauma

5 Suggested Answer

There are several guidelines to follow to make sure ER patients are given the correct priority for their injuries. Trauma patients with unstable vital signs are obviously given the highest priority. Others are assessed by a nurse and seen in order of acuity.

6 1 B 2 A

7 1 complaining of
2 any other symptoms
3 bit of a concern
4 open wound
5 stopped bleeding
6 get an infection

8 Suggested Answer

B: First, there's Melissa Gates, 25. She came in complaining of severe nausea.
A: I see. Does she have any other symptoms?
B: No, but she is pregnant.
A: Okay. That's a bit of a concern, but tell me about the other patient.
B: That's Jack Avery, 8. He has a first-degree burn on his hand.
A: What's the current state of the wound?
B: It's blistered over, but it doesn't seem to be open.
A: All right. Well, we'd better take a look at Jack's hand first. I want to be sure it's not more serious than first-degree.

9 Suggested Answer

Heartland Hospital ER
Patient Triage List

Patient: Melissa Gates, 25.

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Symptoms: Ms. Gates came in complaining of severe nausea. The triage nurse did not identify any other symptoms.
Priority: Ms. Gates's pregnancy is a bit of a concern. However, lacking other symptoms she can wait to be seen.

Unit 11

1 Suggested Answers

1 Infection-causing bacteria are everywhere, and they can spread quickly. Most germs spread through physical contact. Some germs are airborne. Coughing and sneezing can cause the spread of bacteria in the air. In addition, there are many germs on our food such as E. coli.
2 There are many ways to stop infections from spreading. A hand sanitizer can be used to wash your hands frequently. Another way to stop infections from spreading is to maintain good personal hygiene and to keep your surroundings clean.

2 Suggested Answers

1 Use alcohol-based hand sanitizers and wash hands with soap and water.
2 Use diagnostic tests to catch infection early.
3 Brush intensive care unit patients' teeth regularly.
4 Give LPNs checklists to ensure proper fit.

3
1 vigilant
2 alcohol-based
3 drug resistant
4 Hand sanitizers

4 1 B 2 A 3 B 4 B

5 Suggested Answer

You can prevent MRSA from spreading by running diagnostic tests on patients and by washing hands or using a hand sanitizer. Wearing a facemask when coming into contact with a patient being treated for MRSA is also important.

6 1 F 2 F 3 T

7
1 filled with
2 Anything else
3 running high
4 to start
5 right away
6 came back

8 Suggested Answer

B: Well, she complained that her stomach cramps were getting worse.
A: Anything else?
B: Yes, she is running a fever and has been vomiting at least once an hour.
A: I'm afraid it may be E. coli. Keep an eye on her and keep her hydrated. Otherwise this may turn into a kidney infection. And get her on some antibiotics.
B: Right. I'll do that.
A: Has anyone else in this unit been infected?
B: We ran some diagnostic tests on some patients in nearby rooms and her family, but the test results came back negative.

9 Suggested Answer

Patient Name: Karen O'Neil

Condition and symptoms: Patient checked in with E. coli poisoning. Her condition does not seem to be improving. She said that her stomach cramps are getting worse, and she is vomiting frequently. She also has a fever.
Treatment: Keep her hydrated
Concerns: Condition may turn into a kidney infection if not controlled.
Preventative measures: We are running diagnostic tests on patients near Ms. O'Neil and on her family members. We are reminding all hospital staff and visitors to wash their hands or use hand sanitizer frequently. In addition, we are ensuring the hospital cafeteria is cooking all food thoroughly.

Unit 12

1 Suggested Answers

1 Licensed practical nurses check a person's basic health and are often in charge of taking care of any cuts or scrapes that a patient might have. Registered nurses give medications and vaccines, and they also create treatment plans for patients. Nurse practitioners have many of the same tasks as doctors including prescribing medications.
2 Different types of nurses have different levels of education. Licensed practical nurses generally only need a high school diploma and a year of additional educational training. Registered nurses need at least two years of education in nursing, and many hospitals require bachelor's degrees. Nurse practitioners generally have master's degrees. At each level, nurses need to have a license.

2 1 F 2 F 3 T

3
1 C
2 A
3 F
4 E
5 D
6 B

4
1 board-certified
2 registered nurse
3 annual physical
4 vaccine
5 point of entry
6 licensed practical nurse

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5 Suggested Answer

RNs administer vaccines and other medications. They also assist with annual physicals.

6 1 F 2 T 3 F

7

1	RN position	4	too fast-paced
2	ten years	5	electronic health records
3	vaccines	6	switched to

8 Suggested Answer

A: Hello, I'll be interviewing you for the LPN position.
B: Nice to meet you.
A: So what kind of experience do you have?
B: Well, I have worked at Jackson Hospital for the past three years.
A: And why did you go into nursing?
B: I love taking care of people.
A: Excellent. Now, this is a busy hospital. You'll be cleaning lots of wounds and monitoring several patients' conditions.
B: I understand.
A: Is that appealing to you?
B: Yes, I like a fast paced environment.
A: Right, it's definitely fast here. But we still have to treat each case carefully.
B: Of course.
A: Now, have you worked with electronic patient records?
B: Yes, I have used them at Jackson Hospital.

9 Suggested Answer

Interview Assessment
Applicant's Name: Ryan Peterson
Position applied for: Licensed practical nurse

What experience does the applicant have? Ryan has worked in a local hospital for the last three years as a receptionist.
Why did the applicant go into nursing? Ryan says he loves taking care of people.
What tasks is applicant responsible for? Ryan is responsible for managing patient flow and organizing the electronic health records.

Unit 13

1 Suggested Answers

1 Pregnant women need medical services throughout pregnancy They have regular checkups at an OB/GYN clinic. They may also have a c-section if necessary.
2 It is important for older women who are going through menopause to maintain their gynecological health. An OB/GYN can help them deal with the menopause.

2 1 F 2 F 3 F

3

1	F	3	A	5	B	7	G
2	D	4	E	6	C		

4

1	A pregnancy	B	fertility
2	A OB triage	B	OB/GYN
3	A contraction	B	c-section

5 Suggested Answer

The clinic provides many services to both pregnant and non-pregnant woman. It also offers classes for expecting parents as well as fertility treatments. Women going through menopause can make an appointment to receive advice and perhaps medication.

6 1 D 2 A

7

1	be in labor	4	more frequent
2	increase in discharge	5	fifteen minutes apart
3	water broke	6	not quite time for

8 Suggested Answer

B: Hi. I'm experiencing some contractions. I think I might be in labor.
A: Okay. Stay calm. Have you noticed an increase in discharge?
B: No, I don't think so.
A: Do you know if your water broke already?
B: No, not yet.
A: All right. Are the contractions becoming more frequent?
B: No, not really. They've been about twenty minutes apart for quite a while now.
A: Okay. It sounds like you're not in labor just yet. You're just having Braxton-Hicks contractions. They are normal.

9 Suggested Answer

Midwest OB/GYN Clinic
Entering Labor

Signs of Labor: When you enter labor, you will have an increase in vaginal discharge. Your water will break and your contractions will become more frequent.
Braxton-Hicks contractions: These are "false" contractions that are a normal part of late-stage pregnancy. If you have contractions at regular

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intervals without the other signs above, you are likely having Braxton-Hicks contractions. However, it is a good idea to call OB Triage just to make sure.
What to do next: If the call to OB Triage confirms that you are in labor, it is time to go to the hospital. Grab the supplies you prepared for your hospital stay and have someone drive you to OB Triage. Your baby is on its way!

Unit 14

1 Suggested Answers

1 Neurology concerns itself with the intensive study of the nervous system. Neurosurgeons specialize in brain surgery.
2 There are many different kinds of nervous system disorders. Some are relatively mild, such as occasional bouts of headache or vertigo. Severe trauma to the brain can place a person in a coma from which they may never awaken.

2 1 F 2 T 3 F

3 1 C 3 B 5 F
2 A 4 E 6 D

4 1 neurosurgeon
2 neurologist
3 autonomic nervous system
4 central nervous system
5 Neurology
6 peripheral nervous system

5 Suggested Answer

There are many different kinds of nervous system disorders and treatments. Trauma from an accident can lead to paralysis or a coma, which can require neurosurgery. Alzheimer's disease is another disorder affecting the nervous system.

6 1 A 2 C

7 1 Let's get into
2 not sure that I follow
3 a sign of
4 more along the lines of
5 point directly to
6 Paralysis occurs when

8 Suggested Answer

A: We've covered the central nervous system and the peripheral nervous system. Let's get into neurological disorders. What are some signs of trouble in the nervous system?
B: Well, I know that numbness is a big sign.
A: Hmm. That's not always true, you know.
B: I'm not sure that I follow.
A: Let me explain. Numbness CAN be an indicator of a problem with the brain. But it can also be a sign of blood flow problems.
B: Oh, I see. So you're thinking of something more along the lines of a coma.
A: Yes, exactly. That points directly to a problem with the nervous system. But please, explain what a coma is.
B: Sure. A coma is an extended period of unconsciousness when a patient doesn't respond to any stimulus.

9 Suggested Answer

Neurological Disorders:
Vertigo, paralysis, seizures, Alzheimer's

Possible Causes: Vertigo can be caused by neurological disorders or by inner ear problems. However, disorders like paralysis and Alzheimer's point directly to neurological causes.
Parts of the body affected: Vertigo affects the sense of balance. Paralysis affects the muscles. Alzheimer's disease affects a patient physically and mentally.

Unit 15

1 Suggested Answers

1 Many doctors are involved in surgical operations. Surgeons perform the main tasks but other doctors, for example anesthesiologists, are also involved.
2 A variety of tools are used in surgical operations. Scalpels are one of the main tools. Clamps and sutures are also used.

2 1 D 2 D 3 A

3 1 E 3 B 5 F 7 D
2 G 4 A 6 H 8 C

4

1	A	anesthesiologist	B	surgeon
2	A	pre-op	B	post-op
3	A	recovery room	B	operating room
4	A	resection	B	operation

5 Suggested Answer

Surgical patients should know the details of both their pre-op preparation and post-op recovery. Surgeons

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will tell them the details of the procedure, and an anesthesiologist will talk about putting them to sleep.

6 1 T 2 F 3 F

7

1	gall bladder excision	4	should we do
2	take a while	5	push back
3	time for complications	6	operating room

8 Suggested Answer

B: You have an appendectomy on Ms Patel at 10:30.
A: That won't work. Mr. Jenkins kidney transplant is going to be a very long and complex operation.
B: What should I do, Doctor?
A: We may need to push back the appendectomy to later in the day.
B: Okay. There's a slot open at 3:00.

9 Suggested Answer

Dr. Joan Phillips, MD
Surgical Schedule, 4/15

7:00
Patient: Andrew Jenkins
Procedure: Mr. Jenkins will be having a kidney transplant.
Possible Complications: This is a long and complex operation, and Mr. Jenkin's general health is poor.

10:30
Patient: Priti Patel
Procedure: Ms. Patel will be having an appendectomy.
Possible Complications: This procedure should be fairly routine. However, the kidney transplant at 7:00 may run long. Therefore, this operation has been scheduled for noon. Ms. Burns, the nursing staff, and Dr. Gibbons the anesthesiologist have been notified of this schedule change.

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Audioscripts

Unit 1

Director (F): Well, Dr. Matthews, let's talk about your background.
Doctor (M): Certainly, Ma'am. I think you'll find that I'm very well qualified.
Director: I hope so. Let's see, it says here you attended James-Perkins Medical School.
Doctor: That's right. I graduated at the top of my class.
Director: That's excellent. We're always looking for the best and brightest here. What about your residency?
Doctor: I completed an orthopedic surgery residency at St. Margaret's Hospital.
Director: Well, we do need someone with that emphasis. And how long have you been licensed?
Doctor: I passed the medical exam last year, and I've been practicing for the past eight months.
Director: So you don't have a lot of experience in actual practice.
Doctor: No, but I'm a quick learner. And the program at St. Margaret's gave me lots of real-world experience.
Director: Okay, Dr. Matthews. I'll review your qualifications with the board and we'll get back to you this week.

Unit 2

Nurse 1 (M): Hey Wendy, is Mr. Williams due for his medication?
Nurse 2 (F): I don't know. What does his medical record say?
Nurse 1: Well, I'm not sure. I can't read these notes.
Nurse 2: Let me see... Wow, this doesn't make any sense. I think he's supposed to get his meds every day at nine AM, but it doesn't look like he's had any dosages for the last three days.
Nurse 1: Really? That can't be right. How could that happen?
Nurse 2: Wait, I think I see what happened. These notes aren't in chronological order. Someone entered last week's dosages first, after this week's. It's all mixed up.
Nurse 1: So did he get his medication today or not?
Nurse 2: I'm not sure. Who was on duty at nine AM?
Nurse 1: I'll check the shift board ... Tom was here this morning.
Nurse 2: These notes are a mess. We'd better give him a call.
Nurse 1: That's a good idea. We certainly don't want to give Mr. Williams the wrong dosage.

Unit 3

Nurse (M): Dr. Fox, do you have a minute?
Doctor (F): Sure. What is it about?
Nurse: It's about a patient. I'm having trouble communicating with him.
Doctor: What do you mean?
Nurse: Well, he's complaining of constant nausea. I asked about his medical history, but he didn't say much.
Doctor: Is it just a miscommunication? Maybe he doesn't know what you're asking for.
Nurse: It's possible. But I can't tell because he's barely speaking to me.
Doctor: He might just be anxious. It can be scary to feel terrible but not know why.
Nurse: Yeah, he does seem pretty nervous. In fact, I noted that as an additional symptom.
Doctor: Did you use lots of jargon? In my experience, that makes things worse.
Nurse: I might have. Since he has nausea and anxiety, I asked him if he has a history of hypoglycemia.
Doctor: See, that term may not mean much to him. Try "low blood sugar" instead.
Nurse: Good point.
Doctor: Also, make sure to establish a rapport. Even just by asking about his day.
Nurse: Oh. I didn't really do that. I just jumped in with my questions.
Doctor: It's important. It really helps the patient relax.

Unit 4

Doctor 1 (M): Hi, Louise. What are you doing here? I thought your shift ended an hour ago.
Doctor 2 (F): It did, but I have to finish this paperwork. I'm way behind.
Doctor 1: Tell me about it. I came in on my day off last week to finish mine.
Doctor 2: We have too many patients in this department. It's not good for us or for the patients.
Doctor 1: I know, I know. I asked the director if we're going to get another doctor, but she said there's not enough money in the hospital budget to hire anyone.
Doctor 2: The whole situation is terrible. I wish I could take a vacation, but I need the money to pay off my student loans. Besides, if I left, I'd just be even more behind on the caseload when I got back.
Doctor 1: Yeah, I know what you mean. I worry about my kids because I'm hardly ever home anymore.
Doctor 2: That's tough. Well, I'd better get back to this paperwork so I can get some rest tonight. I'm working a double shift tomorrow.
Doctor 1: Sure, Louise. Good luck.

Unit 5

Nurse (M): Good morning, Ms. Lewis.
Patient (F): Hello.
Nurse: Okay, at your last appointment, you said you were getting headaches. Is that still the case?
Patient: No, I'm having knee problems now. The pain is constant.

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Nurse: On a scale of one to ten, how would you rate the intensity of the pain right now?
Patient: Right now, it's moderate. Like a three.
Nurse: And how does it feel at worst?
Patient: At worst it's excruciating. Especially after I exercise. I've had to cut back a lot. I used to run every day.
Nurse: I see. That may be the cause of your knee troubles. Now, can you describe how it feels? Does it burn or throb?
Patient: Mostly the pain is throbbing. Occasionally I get a shooting pain.
Nurse: It goes through your knee?
Patient: No, it extends down into my leg.
Nurse: Let me make a note of that. Last question. How do you treat this pain at home?
Patient: I usually put an ice pack on it. Do you think I'll need surgery?
Nurse: Well, Dr. Samuels will be able to talk to you about that. It'll be just a few minutes while I get him.
Patient: Thanks.

Unit 6

Doctor (F): Mr. Anderson, it's a good thing you came in.
Patient (M): Yeah? I feel pretty awful.
Doctor: Well, it seems that you've come down with endocarditis. Do you know what that is?
Patient: I have no idea, but it sounds serious.
Doctor: It is. Basically, the lining of your heart is inflamed.
Patient: Oh, that's bad. Can we treat it?
Doctor: Yes. I'll start you on some antibiotics immediately. But this could have been avoided.
Patient: Really? How so?
Doctor: Well, you've had a sore throat for quite some time, haven't you?
Patient: Yeah, but I figured it was just a cold.
Doctor: Definitely not. It's caused by the strep virus. Normally, that's an acute condition that we can knock out pretty quickly.
Patient: Okay, but how did that cause my other condition?
Doctor: Untreated, the infection moves to your heart. which it appears to have done. And now we have to get rid of that infection before it causes lifelong complications.
Patient: Lifelong?
Doctor: Yes. Unfortunately, endocarditis can become a chronic condition. It can damage your heart, brain, and liver.

Unit 7

Doctor (F): Well, Mr. Garner, the test results are in. It's just as we suspected, type two diabetes.
Patient (M): Honestly, I'm not surprised.
Doctor: That's right, there's a family history of diabetes.
Patient: Yes, my father had it.
Doctor: I see. You should be pretty familiar with the treatment process then.
Patient: A little. My dad had to take a lot of insulin shots. Will I need to do that?
Doctor: Not necessarily. We'll start with a lifestyle plan and see how that works.
Patient: Okay. We were always buying dad sugar-free candy. Is that what I have to eat now?
Doctor: Actually, no. Small amounts of sweets and chocolate can be part of a healthy meal plan, just like for everyone.
Patient: That's good. Will I have to do a lot of extra exercise to keep my weight down?
Doctor: Not necessarily. You really need the same amount of exercise any healthy person needs.
Patient: Well I can't say I do the recommended amount now. I suppose this is a good reason to start.
Doctor: Absolutely.

Unit 8

Doctor (F): Hello, Mr. Paxton. I'm glad you could come in.
Patient (M): Well, I really want to hear about my cancer treatment options. I'm pretty nervous.
Doctor: That's totally natural. We have a few therapy options available.
Patient: I'm all ears.
Doctor: Great. First, there's always chemotherapy. You would receive regular doses of strong drugs to kill the cancer cells.
Patient: I know a little about chemo. Aren't there some nasty side effects?
Doctor: There can be. You'll likely feel nauseous and lose most of your hair. It can damage some other bodily systems, too.
Patient: That's not very appealing. Is there another option available?
Doctor: Well, at your early stage of cancer, we may be able to try photodynamic therapy.
Patient: I've never heard of that.
Doctor: It's relatively new but has had some good results. We give you a cancer-killing drug that reacts to light. Then we shine a light beam on the tumor, which has absorbed the drug.
Patient: That sounds interesting. What would you recommend?
Doctor: We can certainly try photodynamic therapy. You'll likely need chemo as well, but hopefully not as much.

Unit 9

Doctor (F): Well, Mr. Jameson, I've reviewed your symptoms.
Patient (M): And what do you think?
Doctor: Well, it could be something serious, or it could be something relatively simple.

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Patient: I see. So what's the serious possibility?
Doctor: It could be coronary artery disease.
Patient: Wow. And what's the simple possibility?
Doctor: That it might just be an arrhythmia.
Patient: Shouldn't we be able to tell what I have, just from my symptoms?
Doctor: Well, they have some of the same warning signs. Like the shortness of breath and fast heart rate you've had.
Patient: So what types of treatment am I looking at?
Doctor: It depends on your diagnosis. If it's coronary artery disease, it could be treated with lifestyle changes and drugs. Surgery might be necessary.
Patient: Really? It's that dangerous?
Doctor: Yes, it could lead to a fatal heart attack.
Patient: That's scary. What if it's an arrhythmia?
Doctor: Well, some cases require no treatment at all. Others need medication.
Patient: Then I hope it's just an arrhythmia.
Doctor: Me, too. But we'll have to run a few tests to find out.

Unit 10

Doctor (M): Do we have any serious trauma patients in the ER right now, Sandy?
Nurse (F): Thankfully, no. There are two patients waiting to be seen, however.
Doctor: Okay. Tell me about their conditions.
Nurse: First, there's Carl Henning, 67. He came in complaining of a mild fever.
Doctor: I see. Does he have any other symptoms?
Nurse: None that the triage nurse could identify.
Doctor: That's good. His age is a bit of a concern, though. To be safe, we'll put him on IV fluids. Tell me about the other patient.
Nurse: That's Sarah Crane, 14. She has an open wound on her arm. She got it in a bad bicycle accident, apparently.
Doctor: What's the current state of the wound?
Nurse: It's stopped bleeding but needs to be examined.
Doctor: All right. Well, we'd better take a look at Sarah's arm first. I wouldn't want her to get an infection while waiting.
Nurse: Very well, Doctor.
Doctor: I'm sure she'll need stitches. Please grab a suture kit and meet me there.
Nurse: Sure, I'll be right there.

Unit 11

Doctor (M): Okay. Let's talk about Mr. Harris. He checked in with a staph infection, right?
Nurse (F): Yes, that's right.
Doctor: And how is he doing today?
Nurse: I'm afraid to say his condition is getting worse.
Doctor: Really? How so?
Nurse: Well, the red, swollen wounds on his body are now filled with pus.
Doctor: Anything else?
Nurse: Yes, he said that he was short of breath, and his temperature is running high.
Doctor: I'm afraid it may be MRSA. We need to start him on an aggressive round of antibiotics. And he needs to be in isolation.
Nurse: Right. I'll get started on that right away.
Doctor: Has anyone else in this unit been infected?
Nurse: We ran some diagnostic tests on the other patient in Mr. Harris' room, but his test results came back negative. None of the other patients are exhibiting symptoms.
Doctor: Well, let's keep it that way. Remind everyone to wash their hands or use an alcohol-based hand sanitizer.
Nurse: Of course. And we're disinfecting Mr. Harris' room right now.
Doctor: Good. I'll check on him again before the end of my shift.

Unit 12

Interviewer (M): Jane, I'm Richard Jenson. I'll be interviewing you for the RN position.
Applicant (F): Nice to meet you, Mr. Jenson.
Interviewer: So what kind of experience do you have?
Applicant: Well, I started volunteering in hospitals as a teenager. I've worked as an RN for the past ten years.
Interviewer: And why did you go into nursing?
Applicant: I love taking care of people. It brings me a lot of joy.
Interviewer: Excellent. Now, this is a general practice. You'll be giving lots of vaccines and assisting with annual physicals.
Applicant: Everyday tasks. I understand.
Interviewer: Is that appealing to you?
Applicant: Yes, because there's more interaction with patients. Hospitals are too fast-paced.
Interviewer: Right, it's definitely slower here. But we still have to maintain patient flow.
Applicant: Of course.
Interviewer: Now, have you worked with electronic health records?
Applicant: Yes, we switched to them several years ago.

Unit 13

Nurse (M): OB Triage unit. How can I help you?
Caller (F): Hi. I'm at thirty-eight weeks and I'm experiencing some contractions. I think I might be in labor.

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Nurse: Okay. Stay calm. Have you noticed an increase in discharge?
Caller: Yes, a little bit.
Nurse: Do you know if your water broke already?
Caller: No, not yet.
Nurse: All right. Are the contractions becoming more frequent?
Caller: Yes, they are. They were about fifteen minutes apart, but now they're about every ten minutes.
Nurse: Okay. Those don't sound like Braxton-Hicks contractions. I think you are in the early stages of labor. But it's still not quite time for you to come in.
Caller: Really? It's starting to get painful.
Nurse: Don't worry, that's normal. But we recommend waiting until the contractions are about four minutes apart.
Caller: Oh, okay. Is there anything I need to do until then?
Nurse: Just rest and stay as comfortable as you can. And make sure you have a bag packed with everything you need.
Caller: I've got one.
Nurse: Great. So hang in there and keep timing those contractions.
Caller: Okay. And should I go to OB triage when I'm ready to come in?
Nurse: Yes, you should.

Unit 14

Doctor (F): Okay, so we've covered the central nervous system and the peripheral nervous system. Let's get into neurological disorders. What are some signs of trouble in the nervous system?
Student (M): Well, I know that vertigo is a big sign.
Doctor: Hmm. That's not always true, you know.
Student: I'm not sure that I follow.
Doctor: Let me explain. Vertigo CAN be an indicator of a problem with the brain. But it's often a sign of inner ear problems.
Student: Oh, I see. So you're thinking of something more along the lines of seizures or paralysis.
Doctor: Yes, exactly. Those point directly to a problem with the nervous system. But please, explain what those are.
Student: Sure. Paralysis occurs when a patient loses the ability to move muscles, and seizures result in uncontrolled muscle movement.
Doctor: Good. Now, what's an example of a disorder that doesn't just involve muscle control?
Student: Well, there's Alzheimer's disease.
Doctor: Good example. It leads to physical and mental deterioration.

Unit 15

Doctor (F): Sam, could we go over tomorrow's surgery schedule?
Secretary (M): Sure thing, doctor. First up is a bowel resection for Paul Jackson at 7:00.
Doctor: Ah, yes. Hmm. Is there anything else scheduled for the rest of morning?
Secretary: You are supposed to perform a gall bladder excision on Amanda Burns at 10:00.
Doctor: Right, the cholecystectomy. That could be a problem. I think Mr. Jackson's resection could take a while.
Secretary: Why's that, doctor?
Doctor: I remember the abdominal CT scan showing the tumor to be quite large and advanced. I need to allow time for complications.
Secretary: I see. What should we do, then?
Doctor: Well, we may need to push back that procedure by a couple hours.
Secretary: Okay. Let's see here. There's an operating room open at noon.
Doctor: That would be perfect. Who's the anesthesiologist for that operation?
Secretary: Dr. Gibbons is slated to work with you on that.
Doctor: Good. Please let Dr. Gibbons, Ms. Burns, and the nursing staff know about the change.
Secretary: Will do. See you tomorrow, doctor.

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Medical

Teacher's Book

Career Paths: Medical is a new educational resource for medical professionals who want to improve their English communication in a work environment. Incorporating career-specific vocabulary and contexts, each unit offers step-by-step instruction that immerses students in the four key language components: reading, listening, speaking, and writing. ***Career Paths: Medical*** addresses topics including hospital employees, parts of the body, patient care, common illnesses, and career options.

The series is organized into three levels of difficulty and offers a minimum of 400 vocabulary terms and phrases. Every unit includes a test of reading comprehension, vocabulary, and listening skills, and leads students through written and oral production.

Included Features:

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- 45 reading and listening comprehension checks
- Over 400 vocabulary terms and phrases
- Guided speaking and writing exercises
- Complete glossary of terms and phrases

The **Teacher's book** contains a full answer key and audio scripts.
The **audio CDs** contain all recorded material in American English and British English.

Books 1-3 of ***Career Paths: Medical*** are rated for the Common European Framework of Reference for Languages at A1, A2 and B1 respectively.



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